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SEXUAL ASSAULT AND CONFIDENTIALITY - INDIGENOUS CASE 143

55 year old male with Depression

John is a 55 year old principal of a local school who is referred to you privately, by his general practitioner for further assessment and treatment. He has recently had a myocardial infarction 6 months ago. He is a successful teacher and lives with his wife and three children. He has been trialled on 3 different SSRI's without any significant benefits.

Outline John's causes for treatment resistance and the salient features you would look for in assessment

1. **Risk assessment** : Late onset depression in males can be associated with an elevated risk of suicide. I will evaluate the risk of suicide and keep a high index of suspicion.

2. **Compliance with medication**: He may perceive depression as stigmatising, not willing to engage or seek help, not willing to obtain psychological treatment. There may be negative family attitudes to medication. Side effects of medication (Refer to the factors affecting insight : Personal factors (attitudes to medication and stigma), Medication factors (side effects, complexity of regime, non effectiveness), Illness factors (resistant type of illness, delusions against treating team, lack of trust and System factors (cost, lack of therapeutic alliance)

3. **Incorrect diagnosis**: Bipolar disorder/ Bipolar depression or Major depressive disorder with mixed features . Antidepressants may worsen the long term course of depression and hence this may be a factor. Clues towards bipolarity include (WHIPLASHED): **W**orse or wired with antidepressants, **H**ypomania / mood swings, **I**rritability, **P**sychomotor retardation, **L**oaded family history, **A**brupt termination of depressive episodes, **S**easonal affective syndrome, **H**ypersomnia or Hyperphagia, **E**arly onset of depression and **D**elusions, hallucinations and psychotic features.)

4. **Late onset depression can be more resistant to treatment** as it can be associated with vascular features and white matter hyper intensities. (Vascular Depression Hypothesis). Clues to vascular depression include vascular factors (MI, dyslipidemia, hypertension, diabetes, smoking, TIA's, migraine). The presentation also includes apathy rather than complaints of low mood, agitation or retardation , hypochondriacal and obsessive thoughts, pseudo

dementia). Medical history and Physical examination may reveal Raynaud's, calf claudication etc)

5. **Cognitive dysfunction:** Frontal lobe examination with Luria 3 step test, alternate sequencing, Clock face test. Frontal lobe disease, vascular neuro-cognitive impairment can present with depression as the first presentation.

6. **Drug and Alcohol use disorder:** assess for craving, compulsion, use despite physiological and psychological harm for benzodiazepines, alcohol, amphetamines and opiates

7. **Psychological factors:**

a. **Personality disorder:** Perfectionistic , Narcissistic, Avoidant and Dependant traits may interfere with optimal therapeutic treatment

b. **Adjustment disorder with depressed mood:** Work, financial and relationship stressors

8. **Other medication** such as Beta blockers, diuretics , ACEI are associated with depression

9. **Organic mood disorder:** Thyroid dysfunction, Addison's disease

DEPRESSED MALE WITH AGITATION- MANAGEMENT PRINCIPLES

During your assessment he tells you that he is certain that **he is unable to provide for his family and that they are facing financial ruin.** He is growing increasingly despondent about this. He says he cannot afford to come and see you. His wife finds him pacing about during the day and reports that there is no issue financially and that despite any assurances from work or family, he continues to worry about the future. His wife says that he stays up all night pacing.

Outline the key aspects of your management plan

1. **Risk management:** This depression may be agitated depression that is associated with a high risk of suicide. Therefore I will keep a high index of suspicion and consider a low threshold for admission. This a challenging case as he may not want to consider an admission. If treated in the outpatients then I would arrange close follow up.
2. **Symptoms are suggestive of a more severe form of depression** such a psychotic depression which according to NICE, RANZCP guidelines requires a combination of noradrenergic and dopaminergic antidepressant along with an antipsychotic. The thoughts seem to be nihilistic delusions which along with agitation indicate a possible psychotic depression
3. **Establishing therapeutic alliance** with patient and family is important. Encourage him to attend to discuss possibilities and avoid coercion.
4. If he lacks capacity to make decision for himself due to complete lack of insight and a significant risk of harm exists to himself or family then admission under the mental health act may be needed for assessment and management
5. **Choice of medication:** Choice of medication is complicated by his cardiac involvement. SNRI's such as duloxetine, higher dose venlafaxine can affect the heart as can antipsychotics (QTc prolongation) and a cardiologist involvement will be important. Pre and Post ECG will be required. Initiation of medication may be best done by considering an inpatient admission if he agrees. A risk benefit analysis will be done

6. **Location of admission:** being a principal of a school, admission may need to be considered in another area due to stigma. If he has private insurance he may be able to be admitted in a private hospital.

7. If affective features or mixed features are present he may need additional mood stabilisers such as Valproate or Lamotrigine

8. **Address Drug or Alcohol dependence** by involving the dual diagnosis services. Detoxification if necessary would be best done in an inpatient setting in view of his recent MI

9. **Close liaison with cardiologist** as Heart disease and depression exist in a bi-directional relationship

9. The depression may prove resistant to treat due to the presence of White matter hyper intensities on MRI associated with vascular depression. I will obtain a second opinion and am aware that ECT is an evidenced based treatment in psychotic depression and resistant cases. An informed consent is necessary if patient has capacity otherwise the Mental health act will have to be considered. I will discuss with senior peers

10. Family should be kept involved at all times

1. Risk ASSESSMENT

2. MSE+ Capacity

3. Medication titration + Sleep + therapy

4. Liaison with cardiology

5. Safety planning collaborative plan

6. community support (Family and school) and followup

ETHICAL ISSUES IN REPORT WRITING - CIVIL

John has been reluctantly seeing you monthly. He tells you that the work environment has become unbearable. After an incident at work, **which involved an altercation with students**, John has taken time off due to **sickness**. 5 months later you receive a letter from WorkCover outlining that they need a report from you regarding John. They have also **sent a request for his notes**.

List the key points in your approach to this request

1. **Dual agency dilemma / Conflict of interest** : I will clarify my role. Are you providing a report as a treating doctor or as an independent psychiatrist. (As an independent psychiatrist you will be assisting court whilst as a treating psychiatrist you will act in the best interest of the patient)

2. **Privacy and Confidentiality**: The doctor patient relationship is confidential except in certain circumstances where disclosure is required by law such as **harm to children, harm to third party or harm to self**. In this case, I will seek advice from my **medico-legal insurance** as to my obligations in providing the notes.

3. **Autonomy and informed consent**: I will also speak to the patient and inform him of the request. I will obtain informed consent from the patient after explaining the process and explaining my role.

4. **Beneficence**: **If I were to provide a report will I be acting in the best interest of my patient?**

5. **Non-Maleficence**: If I write a report or provide notes, will my patient come to harm?

Privacy and confidentiality

1. consult the medico-legal cell of the service about the position status
2. consult the indemnity insurance for my obligations
3. notify to the release of information cell after obtaining the consent of the patient
4. Seek advice from the service head or peer group

MANAGEMENT OF PATIENT SUICIDE

John's mental state deteriorates over the next few months. You admit him to a private hospital. 6 days into the admission you receive a phone call from the ward stating that John has committed suicide.

Outline how you would proceed

1. **Attend ward and gather information:** How, when, who witnessed it. Have police been informed? Has the clinical director and NUM been informed?
2. **Advise staff to isolate the body** and not disturb the scene until police arrive.
3. **Inform** clinical director and NUM personally
4. **Document** last mental state, ask staff to ensure adequate documentation. Photocopy notes and keep one copy as it will be required for the coroner
5. **Inform family:** This is best done after discussing it with staff and director as to the best way to inform family. A face to face meeting with a staff member that the family knew would be appropriate. Respect family wishes.
6. **Offer the family opportunity to ask questions, vent anger and hostility. If family wishes to complain, explain avenues to do this i.e complaints pathway.**
7. **Inform family of supports** available and identify any vulnerable individuals in the family that may need additional support
8. **Staff debriefing**
9. **Patient debriefing** to avoid rumours spreading and to maintain trust in the therapeutic relationship.
10. **Contact medicolegal insurance** and inform them
11. **Coroner's report** preparation will be necessary once a request is obtained
12. **Psychological autopsy / Root Cause Analysis (RCA)**
13. **Self reflection:** speak to colleagues and obtain advice

14. **Case conference** to learn from incident

Root Cause Analysis (RCA)/ Psychological Autopsy - Position Statement RANZCP

Purpose:

RCA is a systematic, retrospective process used to identify the underlying factors that contribute to an adverse event or near miss. Its aim is not merely to address the immediate error but to uncover latent system failures and process breakdowns.

Approach:

Involves a detailed investigation using techniques such as the “5 Whys” or fishbone diagrams.

Collects data through interviews, record reviews, and observations to move beyond surface-level explanations.

Focuses on understanding how and why an incident occurred so that targeted, sustainable interventions can be implemented.

Outcome:

Provides actionable recommendations to prevent recurrence, thereby driving quality improvement and enhanced patient safety.

SAC is a preliminary classification system used at the point of incident reporting to determine the level of risk.

DEBT AND MENTAL ILLNESS- PRINCIPLES

1. **Clarify role:** Your role is not to provide financial advice but to refer patients to appropriate individuals and liaise with organisations to assist the patient
2. Advise patient not to ignore the problem
3. Identify priority debts and advise patient to contact and inform respective organisations e.g banks, utility companies
3. Is the client thinking of borrowing more money?
4. Is there a mental illness that may be impacting on financial stability? e.g. mania, gambling, substance use disorder
5. Involve social worker
6. Maximise client government benefits
7. Refer to a debt advisor or financial advisor
8. Refer to legal aid if legal issues present.
9. Support letters can be sent to bodies advocating for flexibility e.g interest only payments on mortgage etc.
10. Involve consumer consultant

YOUNG MALE WITH SCHIZOPHRENIA- ASSESSMENT OF RISK

Stan is a 20 year old indigenous male. He has been admitted to the mental health unit after a violent assault on his father. His parents report that he has not been himself over the last 6 months and has been seeing talking to himself on a number of occasions. He is also reported to smoke cannabis on a regular basis.

List the short and long term risk factors you would assess for in this case that would increase the risk of violence?

This question can be answered by using the HCR-20 in the diagram.

Additionally one must add the cultural aspects that play a role.

1. Holistic model of health
2. Distrust of 'Western mental health practices'
3. Lack of availability of indigenous mental health liaison workers
4. Geographical isolation and poor follow up
5. Negative family attitudes to medication and societal stigma
6. Specific issues related to non-compliance as outlined in notes.

YOUNG INDIGENOUS MALE WITH SCHIZOPHRENIA-ONGOING SYMPTOMS

Following your assessment of Stan an indigenous male, you diagnose him with schizophrenia. His medical investigations are normal. You find out that he has been admitted previously to another mental health unit where he was diagnosed with schizophrenia. He was started on Risperidone 6 mg, which he stopped 9 months ago. He continues to complain of ongoing auditory hallucinations but refuses to take any medication.

Outline your approach to this situation.

List the factors affecting compliance in this case?

Outline how you would specifically address his resistant hallucinations after finding out he was compliant with medication.

1. Attempt to establish therapeutic alliance and assess for factors affecting insight and compliance

a. Personal factors

- Cultural aspects (holistic understanding of health, alternate explanatory models of illness)
- negative attitudes to medication
- Stigma
- Familial negative attitudes ot mental illness

b. Medication factors

- Side effects of medication: EPSE's (AIMS examination), High prolactin levels and sexual side effects, gynaecomastia, weight gain, sedation and slowing down
- Difficulty remembering to take medication
- Ineffectiveness of medication

c. Illness factors:

- Delusions and hallucinations regarding treatment
- Personality factors with lack of trust
- Cognitive impairment
- Substance use

d. System factors:

- Lack of cultural sensitivity
- Absence of culturally trained workers and absence of indigenous mental health liaison workers
- Distance to travel to see team
- Cost

Treatment of resistant hallucinations

1. Provide psychoeducation to patient about treatment options and attempt to engage him
2. Rule out organicity: MRI scan , TFT's
3. Rule out substance use
4. Switch antipsychotics to alternate one like Olanzapine or Asenapine
5. If non-compliance is an issue consider depot preparations. Issues such as consent and legal framework are to be considered. e.g CTO
6. If no response , discuss possibility of clozapine with patient and family with risks explained. Care to be taken in view of ethnic profile as higher risk of metabolic issues.
7. CBT and hearing voices group taking into account cultural aspects or one on one therapy tailored to patient incorporating cultural aspects and nature of voices. A particularly skilled therapist will be required to achieve this. involve the indigenous liaison worker.
8. Augmentation strategies like mood stabiliser augmentation with antipsychotic
9. Music and headphones for distraction
10. Anxiety reduction strategies
11. TMS has some evidence but is not routine.

STIGMA AND MENTAL HEALTH - COMMUNITY MANAGEMENT

After a 4 week admission Stan is better. The family is concerned about his being discharged in the community as it is an indigenous community and his parents are worried about the impact of this on Stan's mental health. They do not want any community follow up from the CMHT, but want to go to the traditional healer.

How would you manage this situation?

1. Arrange a meeting with parents and Stan: Involve a indigenous liaison worker if possible.
2. Clarify specific concerns of the family: What do they feel will happen? How do they feel we can help? What do they think they would like?
3. Provide psycho-education about schizophrenia and its treatment options. Attempt to establish therapeutic alliance by taking into account cultural issues
4. Explain that traditional healing systems and western models can co-exist side by side: We can work together with the traditional healer in a liaison model
5. Consider a meeting with community elders after obtaining consent from family and Stan.
6. Reassurance provided in a realistic manner about role of mental health services: explain the nature of the team and community follow up. Also explain that if risk issues are present such as risk of harm to self or others legal aspects may be needed such as CTO or inpatient admission to protect Stan. At all points explain to the family that you will be acting in the best interest of Stan and Family.
7. If media is involved than involve the service media liaison officer
8. Wider issues such as community education, combating stigma, employment for Stan.

MOTIVATIONAL INTERVIEWING PRINCIPLES

You have an obese patient with depression and diabetes. The endocrinologist has reported that your patient has to lose weight to prevent the onset of Insulin Dependant Diabetes Mellitus. During the interview when you bring up the topic of losing weight, She says, “ I don’t need to follow my diet as long as I’m feeling OK.”

Outline your approach to the situation to assist the patient in losing weight.

(Please note that the same principles are applicable for Alcohol and the example used here is for alcohol dependence).

Interviewing Principles (OARS)

1. Assess for stage of change

2. Ask Open-ended questions

- I understand you have some concerns about your drinking (losing weight). Can you tell me about them?

3. Make Affirmations

- The patient’s strengths and efforts for change are noticed and affirmed
- I appreciate that it took a lot of courage for you to discuss your drinking (weight) with me today
- You appear to have a lot of resourcefulness to have coped with these difficulties for the past few years
- Thank you for hanging in there with me. I appreciate this is not easy for you to hear

4. Use Reflections to enhance cognitive dissonance

- Involves rephrasing a statement to capture the implicit meaning and feeling of a patient’s statement
- Encourages continual personal exploration and helps people understand their motivations more fully
- You enjoy the effects of alcohol in terms of how it helps you unwind after a stressful day at work and helps you interact with friends without

being too self-conscious. But you are beginning to worry about the impact drinking is having on your health. In fact, until recently you weren't too worried about how much you drank because you thought you had it under control. Then you found out your health has been affected and your partner said a few things that have made you doubt that alcohol is helping you at all

- I understand that you feel OK and have not considered losing weight. I acknowledge that it is a difficult topic for you to discuss but I want to touch on the impact of your weight on your health. Is that OK? (Since patient is at precontemplation).
- What are the advantages of not losing weight? What are the disadvantages?

5. Use Summarising

- Links discussions and 'checks in' with the patient
- If it is okay with you, just let me check that I understand everything that we've been discussing so far. You have been worrying about how much you've been drinking in recent months because you recognise that you have experienced some health issues associated with your alcohol intake, and you've had some feedback from your partner that she isn't happy with how much you're drinking. But the few times you've tried to stop drinking have not been easy, and you are worried that you can't stop. How am I doing?

4 further principles

1. Express Empathy
2. Roll with resistance
3. Enhance self efficacy
4. Develop discrepancy or enhance cognitive dissonance

Reference: RACGP

TACKLING NON-ADHERENCE DUE TO FORGETFULNESS

A 52-year-old female who takes 2 antidepressants, 2 mood stabilisers and 2 antipsychotics along with metformin and anti-hypertensive reports that she finding it difficult to remember to take her medication.

List the strategies to assist her

1. Dosette box
2. Alarm clock, Mobile phone alarms
3. Post it's
4. Simplify regime e.g. all morning or night if possible
5. Involve family member in administration of medication
6. If lack of insight- adherence therapy incorporating motivational interviewing techniques
7. Depot medication
8. Rule out cognitive dysfunction and treat cause if possible

EATING DISORDERS- RISK FACTORS

A 21 year old female has been recently diagnosed with an eating disorder.

List the factors you would focus on in developing your formulation

- Female gender
- Being from the developed world where the 'thin ideal' prevails.
- Migrants from the developing world
- Those living in urban areas and undertaking life pursuits where body image concerns predominate, for example, competitive gymnastics and fashion modelling.
- Genetic predisposition
- Early menarche
- Epigenetic changes to DNA structure: food deprivation, severe trauma
- Family history of eating disorders
- Early attachment and developmental difficulties
- Premorbid obesity
- Interpersonal problems
- Dieting or other causes of rapid weight loss (Mitchison and Hay, 2014; Stice, 2002).
- Rapid weight loss from any cause, including physical illness, can trigger cognitive changes

Psychological factors

- A 'milieu' of weight concern in formative developmental years
- Low self-esteem (all eating disorders)
- High levels of clinical perfectionism for those with anorexia nervosa
- Impulsivity for bulimic disorders.
- Emotional and sexual child abuse increases personal vulnerability, most likely through impeding a robust sense of self-worth and adaptive coping. The eating disorder then provides a sense of improved self-esteem and self-control for the individual (Stice, 2002).
- Obsessive thinking about food, in turn precipitating and perpetuating the symptoms of anorexia nervosa (Keys et al., 1950).
- Positive reinforcement as maintaining factor e.g sense of achievement, societal reinforcement

Reference: Hay, P., Chinn, D., Forbes, D., Madden, S., Newton, R., Sugenor, L., ... & Ward, W. (2014). Royal Australian and New Zealand College of Psychiatrists

EB+ CSA= Personal vulnerability (loss of self esteem+ self control) ---> Regain via eating

clinical practice guidelines for the treatment of eating disorders. Australian and New Zealand Journal of Psychiatry, 48(11), 977- 1008.

EATING DISORDER - ASSESSMENT

Jane is a 16-year-old female referred by her GP. She lives with her parents and younger sister. She presented to the GP with nausea and vomiting. Her medical investigations revealed a low potassium. The GP feels that she may have an eating disorder.

List the salient features in history, examination and formulation you would focus on to make a diagnosis

1. History Taking: Symptoms of anorexia nervosa which include but are not limited to: dietary restriction; weight loss; inability to restore weight; body image disturbance; fears about weight gain; bingeing; purging; excessive exercise; early satiety; constipation; and the use of laxatives, diuretics, or medications to lose or maintain low weight (APA, 2013). Symptoms of bulimia include preoccupation with food and compensatory behaviours after binges.

2. Disturbed eating behaviours, e.g. eating apart from others and ritualistic patterns of eating such as prolonged meal times and division of food into very small pieces (Wilson et al., 1985).

3. Accurately assess nutritional and fluid intake: Specific enquiries made as to the adequacy of main meals and snacks consumed.

4. Collateral: Collateral sources such as family members and other clinicians involved in the person's care should be utilised.

5. A brief physical examination including measurement of weight, height, calculation of BMI, seated and standing pulse rate to detect resting bradycardia and/or tachycardia on minimal exertion due to cardiac deconditioning, blood pressure (seated and standing) and temperature. These findings are needed to determine if immediate hospital admission is required. The assessment should also include any history of fainting, light-headedness, palpitations, chest pain, shortness of breath, ankle swelling, weakness, tiredness and amenorrhoea or irregular menses. Check nails, swollen glands, dental caries etc.

6. Investigations: Serum biochemistry to detect hypokalaemia, metabolic alkalosis or acidosis, hypoglycaemia, hypophosphataemia, and hypomagnesaemia, serum liver function tests, serum prealbumin levels and a

full blood examination looking for evidence of starvation-induced bone marrow suppression such as neutropaenia and an electrocardiogram (ECG). A bone mineral density scan should be performed routinely if the person has been underweight for six months or longer with or without amenorrhea and thereafter every two years whilst still struggling with an eating disorder (Mehler et al., 2011).

7. Assessing psychiatric comorbidity: e.g. anxiety, depression, substance misuse, suicidality, personality disorders, anxiety disorders and deliberate self-harm. However, clinicians should be aware that depression, obsessional thinking, anxiety and other psychiatric symptoms can represent the reversible effects of starvation on the brain (Keys et al., 1950).

8. Cognitive Assessment: Assessing cognitive changes due to starvation such as slowed thought processing, impaired short-term memory, reduced cognitive flexibility and concentration and attention difficulties (Hatch et al., 2010).

9. Possible predisposing and precipitating factors: see risk factors in previous question

If the question asked approach to assessment, risk assessment would be important and that would include stating that a low threshold for admission required if low pulse and BP with ECG and electrolyte changes.

ANOREXIA NERVOSA - CHALLENGING MANAGEMENT

Following your assessment you make a diagnosis of Anorexia Nervosa. She has a BMI of 14. On the paediatric ward where she has been admitted, she refuses to eat and threatens to kill herself if given food. The nursing staff has become hostile against the patient. They ask you to have her transferred to the psychiatric ward.

Outline your immediate management plan?

1. **Ethical issue** : Duty of care vs. Autonomy of patient. Since she is 16 she would have a guardian. Find out who are legal guardians.

2. **Arrange meeting with nursing staff with CL nurse:** Acknowledge the difficulty in management and provide positive affirmation. Inform that team will be available to support them

3. **Inform paediatric team about the seriousness of the issue.** BMI of 14 may indicate serious medical risks and a thorough medical evaluation and treatment to avoid refeeding syndrome is required and this is best provided on the medical ward as regular monitoring of vitals is required.

4. Provide psycho-education about Anorexia nervosa, personality factors and psychodynamic mechanisms resulting in anger or refusal of food. (projection, acting out, splitting, anger turned inwards etc.)

5. Arrange a nurse special either a medical nurse special or psychiatric nurse special.

6. Provide ongoing psychiatric liaison

7. Inform guardian respecting confidentiality as maintaining confidentiality may be required to build trust and establish therapeutic alliance

8. Involve the specialist eating disorder service

9. If she refuses to eat with significant medical risks, speak to guardians, clinical director/ emergency guardianship tribunal to obtain advice about treatment and use of mental health act. This may differ from state to state and country to country. .

ANOREXIA NERVOSA-PSYCHOEDUCATION TO FAMILY

She continues to remain on the paediatric ward. Her behaviour has settled. Her parents come to meet you to discuss management of their daughter's condition. They feel that they are to blame for her condition.

Outline the key points you would cover in the discussion?

1. Establish therapeutic alliance: Explain the Anorexia Nervosa is an illness and that they are not to blame for this. Provide praise, encouragement and positive reinforcement as they will be important part of treatment.

There is no research evidence that proves a link between family dysfunction and the onset of anorexia nervosa, although research supports improved outcomes in anorexia nervosa when families are involved in treatment.

2. Provide opportunity to ask questions first

RCT+ NICE

3. Explain that anorexia is a eating disorder:

Anorexia nervosa occurs in about 0.5% of girls and young women in developed societies. 9 out of 10 individuals affected are females.

4. Explain the seriousness of the illness but at the same time explain that it is treatable.

It can lead to severe weight loss, chronic physical disabilities such as osteoporosis (bone loss with weakened bone structure and increased risk of fractures), growth retardation, infertility, impaired thinking and concentration, bowel and intestinal disorders and major disruptions to emotional, social and educational development. It can be life threatening.

People can fully recover from anorexia nervosa. Research says that if anorexia nervosa is treated early in its course, particularly in children and adolescents, then recovery occurs more quickly and more often than if treatment is delayed

5. Weight restoration is an essential first step in recovery but not a sign of recovery. It takes time to heal the mind from anorexia nervosa and restore a sufferer to a state where they are not dominated by negative feelings related to weight and food.

6. Explain that management will be as part of a **multidisciplinary team** with a psychologist, nurse, psychiatrist, dietician and family therapy

7. **Psychological therapies:** Inform that families play an important role in management and research shows that family therapy (e.g. Maudsley Family therapy) is evidence based for the treatment of anorexia nervosa.

(See components of family therapy in the next section)

8. **Medication:** Medication may be required if the obsessive thoughts are severe or if there is anxiety and depression. SSRI's (antidepressants) and antipsychotics such as olanzapine are indicated.

9. Provide information about support groups

Medical concerns

1. Electrolyte imbalance
2. Osteoporosis
3. Infertility
4. growth retardation
5. Cardiac issues
6. Alter bowel/ gut health - gut brain axis

Psychological

1. Impaired attention concentration
2. Poor academic performance
3. depression
4. anxiety
5. OCD
6. increased self harm

PEER SUPPORT + Carer support via national wide Butterfly foundation

Online & in-person support groups: for individuals and carers

Peer mentoring program

Recovery programs, psychoeducation, and community webinars

Peer Role: Trained lived experience peer mentors offer structured support across stages of recovery

Peer support workers are individuals with lived experience of an eating disorder or being a carer who provide emotional, informational, and practical support to others on their recovery journey.

PRINCIPLES OF FAMILY BASED TREATMENT IN ANOREXIA NERVOSA

1. Adolescent is embedded in the family and the involvement of the family is a key component of success.

Phase 1 : Weight restoration: Therapist has a direct observation of the family's interaction patterns during eating. Therapist directs discussion (enhance circular communication) to reinforce a strong alliance between parents at promoting weight restoration in their child.

Phase 2: Transitioning control of eating back to the adolescent

Phase 3 : Adolescent issues and termination: After medical issues are addressed and patient has regained a normal weight. Enhancing separation individuation, addressing conflict, enhancing self efficacy.

Other principles such as addressing high emotional expression, psychoeducation, allow family to vent anger and hostility, take feeling of blame away from parents remains.

Phase 1 Goals

Parents take full control of the adolescent's eating and weight restoration.
Remove blame from both the child and family (externalise the illness).
Ensure medical safety through regular monitoring

Phase 2 transition Returning Control to the Adolescent (Sessions 11–16)

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@cegl cvnjmp_rgm1 md c k mrgml_j_lb amelgrgtc gqscq_q _nnpmnpg_rc,

Guide the family in negotiating a controlled handover.
Support adolescent in gaining confidence and responsibility.

REFEEDING SYNDROME-PRINCIPLES IN MANAGEMENT

Refeeding syndrome can occur in Anorexia nervosa, chronic alcoholism, oncology patients, severe depression and anyone with severe weight loss.

Assessment

- Dietician assessment
- Assess weight change, nutrition, alcohol intake and psychosocial issues
- Investigations: PO₄, Mg, NA and K
- Glucose

Correct deficiencies with close monitoring with admission to a medical ward.

Slow replenishment

Refeeding Syndrome (RANZCP Guidelines, 2014)

- Refeeding syndrome is understood to be due to the switch from fasting gluconeogenesis to carbohydrate-induced insulin release triggering rapid intracellular uptake of potassium, phosphate and magnesium into cells to metabolise carbohydrates (Kohn et al., 2011).
- The low body stores of such electrolytes due to starvation, can lead to rapid onset of hypophosphataemia, hypomagnesaemia and hypokalaemia. In addition, insulin-triggered rebound hypoglycaemia can occur, exacerbated by the fact that such patients have depleted glycogen stores.
- Risk of refeeding syndrome can be reduced by 'starting low' and 'going slow' with nutrition, and monitoring serum phosphate, potassium and magnesium daily for the first 1–2 weeks of refeeding, and replacing these electrolytes immediately if they fall below normal range
- The CPG group recommends taking a 'middle path' with adults, commencing refeeding at 6000kJ/day. This should be increased by 2000kJ/day every 2–3 days until an adequate intake to meet the person's needs for weight restoration is reached.
- This diet should be supplemented by phosphate at 500mg twice daily and thiamine at least 100mg daily for the first week, and thereafter as clinically indicated for people at high risk of refeeding syndrome (e.g. BMI <13).
- For people at high risk of refeeding syndrome, commencing with continuous nasogastric feeding with low-carbohydrate preparations (i.e. 40–50% of energy from carbohydrates) seems

prudent to avoid triggering postprandial rebound hypoglycaemia due to insulin secretion in people with inadequate glycogen stores.

RANZCP Eating Disorder Guidelines

RISK FACTORS IN THE DEVELOPMENT OF PTSD

A 71 year old Vietnam Veteran has long standing history of PTSD. He reported a traumatic upbringing with sexual abuse.

List the main factors that would explain the development of PTSD (including others not in the vignette)

1. Pre-trauma:

- History of trauma
- Life stressors
- Poor coping skills
- Low self esteem
- Genetic vulnerability
- Brain structure - decreased hippocampal volume
- Gender: Women
- Poor family dynamics
- Family history of anxiety

2. Peri-trauma

- Traumatic event associated with dissociation or belief where they feel they are responsible
- Severity of trauma
- Proximity of trauma
- Type of trauma and nature of trauma (sudden, unexpected, enduring and recurring)

3. Post Trauma Factors:

- Lack of social support
- Blaming the victim
- Secondary victimisation
- Lack of treatment

TREATMENT STRATEGIES FOR PTSD

Psychological therapy:

- Trauma focussed CBT or EMDR is the treatment of choice for PTSD:
- Trauma focused CBT included: In vivo and imaginal exposure
- Cognitive therapy or cognitive processing therapy

2. **Medication** should not be used as routine 1st line in preference to TF psychological treatment

Where prescribed:

- SSRIs
- Other newer antidepressants: tricyclics, phenelzine

Other supportive strategies:

- Enable patient to confront distressing memories
- Manage avoidance responses
- Reduce and manage associated arousal & distress

Stages of CBT:

- Stabilisation and engagement
- Education and information
- Anxiety management
- Trauma exposure
- Cognitive restructuring
- Relapse prevention and maintenance

Important: PTSD is co-morbid until proven otherwise (RANZCP)

KEY POINTS IN ASSESSMENT OF PTSD

PTSD is co-morbid unless proven otherwise

1. Ensure a Full History

- Trauma
- Current Symptoms, usually bunched by morbidity
- Past Treatment ? Evidence Based, especially psychotherapy
- If not trauma-focussed, why not?
- Family and developmental history

2. Who were they before PTSD set in? Functioning, goals etc.

3. What risk factors were present?

4. Current Circumstances – Relationships, Compensation

5. Substance Abuse

6. Forensic history

7. Mental State Examination

- Signs of Arousal, Affect, Thought Content

8. Impact of PTSD on psychosocial aspects

Management Plan

- Trust and Engagement
- Stabilisation of Comorbidities
- Anxiety and Anger Management
- Specific Psychotherapies
- Pharmacotherapy
- Functional Rehabilitation

INTELLECTUAL DISABILITY, SCHIZOPHRENIA AND EPILEPSY - CONSULTATION LIAISON

Maria, aged 25 years, has a history of epilepsy since early childhood with multiple seizure types, a poor response to anticonvulsants, and longstanding cognitive difficulties (her full-scale IQ is 65). She lives with her parents in a small country town, and intermittently attends a supported work program packing boxes of fruit. She was admitted in status epilepticus to a tertiary intensive care unit and was ventilated for 3 days. Two days ago, she was transferred to the neurology ward. Since waking up she has been telling her parents that “everyone is spying on me with satellite cameras”, and that the nursing staff are saying horrible things about her. She is worried that the food is poisoned.

You are asked by the neurology team to see Maria.

Question

Outline the information you would seek before seeing Maria.

1. Risk Assessment:

- Risk of harm to self and others
- Risk of absconding
- Risk of accidental self-injury
- Current environment : observation level, 1 on 1 special , single room etc

2. Evidence for delirium:

- Acute onset, fluctuating cognition, reversal of sleep wake cycle.
- EEG and seizure status- is she seizure free?
- Blood tests : Electrolyte abnormalities, Infections (CRP and ESR with leucocytosis), Urine infection, Liver or renal abnormalities. MRI or CT scan to rule out focal lesions.
- Alcohol use and Drug use: LFT and GGT for evidence of alcohol misuse as Alcohol withdrawal can present with delirium tremens the onset of which begins on 72 hours. History of alcohol or drug use.

3. Evidence of psychosis:

- In addition to delusions, is she complaining of auditory hallucinations, passivity phenomenon which could indicate schizophrenia.
- Is there a past history of psychosis – previous notes?

- Medications: Is she on antipsychotics? Antipsychotics can reduce seizure threshold
- Is she on anti epileptics that can worsen psychosis such as Levatiracetam, which is associated with risk of psychosis.
- Is the dose stable or has that been changed.
- What has been her pre morbid functioning?

4. Evidence of Peri Ictal, Ictal or Post Ictal psychosis:

- Temporality of symptoms to seizure activity
- Last EEG and presence or absence of epileptic activity
- Medications: Serum Levels of anti epileptics
- Forced normalisation: Has the normalisation of the EEG unmasked psychotic symptoms?
- Neurologist opinion.

INTELLECTUAL DISABILITY AND EPILEPSY IN CONSULTATION-LIAISON

What the main differential diagnoses?

1. Peri-Ictal psychosis
2. Post Ictal psychosis
3. Delirium - give causes e.g electrolyte disturbances, infection etc as discussed in previous question
4. Drug induced - antipsychotics / antiepileptics
5. Psychosis due to hypoxia, encephalopathy
6. Exacerbation of underlying psychotic illness
7. Environmental reasons - misinterpretation of environmental cues
8. Cognitive deficits

INTELLECTUAL DISABILITY, SCHIZOPHRENIA AND EPILEPSY - CONSULTATION

LIAISON

Maria experiences another seizure and is now under observation in the neurology ward. Maria attempts to abscond from the ward on a number of occasions and the nursing staff call you as her parents have arrived and want to take Maria home. They believe that the treatment is 'not doing her any good'. The nursing staff call you for advice.

Question

Outline your immediate management plan

- 1. Attend the ward** as soon as possible to clarify the situation
- 2. Manage risk** by ensuring that Maria has one on one nurse special until the situation is addressed.
- 3. Challenging situation** as it is difficult to clarify whether the issue is due to delirium or medical condition which will require treatment under duty of care. If doubts are present obtain advice from emergency guardianship as part of good practice and involve clinical director and neurologist.
- 4. Arrange a family meeting** (case conference with neurology team) to provide psychoeducation to the family about the treatment and risks involved. Risk of seizure may be high and Maria would need to remain in hospital. Provide positive reaffirmation, reassurance about the role of the teams. Attempt to engage family. Inform family of legal obligations as part of treatment.
- 5. Ethical dilemma:** Parents may be guardians if Maria is significantly intellectually disabled. They may have the legal right to make decisions for Maria but if there is a significant risk of harm to Maria, the medical team has a duty of care towards Maria. Obtain emergency guardianship advice or medico legal advice.
- 6.** If Maria is experiencing delirium, then mental health act cannot be used and treatment will proceed under duty of care. If she is experiencing mental illness symptoms impacting on her safety mental health act may be needed.

7. Medication use in the context of behavioural disturbance should be a cautious use of antipsychotics and benzodiazepines in small doses as organic brain disorders increase significant risks of NMS, paradoxical disinhibition and hence treatment should proceed on the medical ward.

8. Involve clinical director and Nurse Unit Manager

INTELLECTUAL DISABILITY, SCHIZOPHRENIA AND EPILEPSY - CONSULTATION

LIAISON

The neurologist is happy for Maria to remain under her care as the risk of further seizures is quite high. However, the nursing ward staff have now become anxious about managing Maria on the neurology ward because they feel she is 'crazy' and may be a danger to the other sick patients.

Question: Describe how you would address the nursing staff's concerns.

1. Therapeutic alliance (Collaborator): Attempt to establish a therapeutic alliance with nursing staff. Provide encouragement, positive affirmation, acknowledge the difficult situation and offer support from mental health team.

2. Psychoeducation to nursing staff (Educator): Explain that Maria is not 'crazy' but is experiencing a medical illness that may impact on her behaviour. The illness is treatable and that the condition may be life threatening and serious and hence monitoring on a medical ward is important.

3. Advise on management strategies for behavioural disturbance. (Clinician)

a. Environmental modification:

- Provide a separate room
- 1 on 1 nursing (mental health nurse or general nurse)
- Quiet area and avoid excessive stimulation

b. Medication intervention:

Avoid medication if not needed as organic disorder may increase the risk of side effects. Antipsychotics and benzodiazepines are indicated in aggression but have to be used cautiously in Maria as individuals with intellectual disability are susceptible to side effects such as EPSE, NMS and paradoxical disinhibition and therefore small doses will be required. Treatment of delirium is necessary.

c. Psychological strategies

- Supportive counselling
- Use audio visual aids in communication

- Consistent nursing staff as much as possible
- Involve family if supportive
- Speak in soft voice and avoid jargon
- Reorientation of room and other areas. (clear demarcation with signs)

4. Involve the C-L nurse to provide specific strategies on managing behavioural disturbance

5. Arranging ongoing reviews and case conferences with the neurology team to provide support. (Involve family; they may stay with her, bring food as she thinks food is poisoned. Address point of them coming from a small country town)

OPIOID ASSESSMENT

You are a psychiatric registrar working in the Drug and alcohol service. Rhonda is a 35 year old female with 3 children. She lives with her partner who is 37. She has a history of Drug and Alcohol misuse since the age of 19. She had been a heavy user of cannabis, alcohol, and heroin. She continues to use heroin which she buys off the streets.

Question:

Describe your assessment of Rhonda

1. Risk assessment : Physical examination to rule out opioid intoxication. Confusion, pin point pupils, sedation. Pulse, BP and respiratory rate to rule out overdose. (Unconscious , Pinpoint pupils Slow, shallow respirations; respirations below 10 per minute Pulse rate below 40 per minute Overdose triad: apnea, coma, pinpoint pupils) **Risk of harm to children. Where are children currently?**

2. Interview approach : Non-judgemental attitude with reflective listening

3. Substance use history (Heroin and other substances):

a. age of first use; substances used; change in effects over time; history of tolerance, overdose, withdrawal; attempts to quit; current problems with compulsivity or cravings.

b. addiction treatment history; e.g., previous treatments for addiction, types of treatments tried, outcomes of treatment attempts, reasons for relapse.

c. Mode of use, same dealer or different dealer, sharing needles, use on own or with other people, expenditure on heroin.

4. Psychiatric history: Substance misuse is often co-morbid and on MSE I will look for :

- Evidence of depression
- Bipolar disorder
- Psychosis
- Eating disorders
- Personality dysfunction: borderline traits

5. Family history : Family history of mental illness and substance use

6. Medical history : History of hepatitis B and C, vaccinations for Hep B and C, history of STD's and HIV. Heart disease such as pericarditis, endocarditis or valvular disease.

7. Cognitive examination: Any frontal lobe deficits as can impede detoxification and rehabilitation attempts.

8. Physical Examination:

- Needle marks
- Skin abscesses
- Cellulitis
- Jaundice
- Poor nutrition and hygiene

7. Psychosocial history:

- quality of recovery environment
- family/living environment (accommodation)
- substance use by members of support network (relationships)
- financial difficulties
- Child development : physical, cognitive and emotional development

8. Readiness to change:

- Patient's understanding of his or her substance use problem
- Stage of Change the patient is in
- Patient's interest in treatment now
- Whether treatment is coerced or voluntary

PARENTING CAPACITY ASSESSMENT

You are contacted by the Department of human services to provide a report on Rhonda's capacity to look after her children.

List the key principles in approaching this request and what are the key components of a parenting assessment?

1. Clarify request and obtain request in writing: The request can pose a dual agency dilemma as the role of a treating psychiatrist can conflict against one of an independent psychiatrist. Are you in the best position to write this report or should this be delegated to a psychiatrist familiar with family court. Discuss with director or senior colleague.

2. Awareness that presence of a mental illness does not always indicate significant risk to the children

3. **Confidentiality** is overridden in cases where there is risk of harm to children and there is a duty to warn.

4. Principles in gathering information

Information should be gathered from multiple sources e.g home visits, multiple assessments, observation, history, tests and rating scales. Children should be seen on their own and with parents and both parents should also be interviewed independently. Parent -child observation is important part of assessment.

5. Assessment of child

- What proportion of time does the child spend with parent with mental illness?
- What is the nature of the child's attachment with parent ? Secure / insecure avoidant/ secure ambivalent / disorganised
- Disruptions from parent
- Child's experience of care
- Age of child and developmental difficulties
- What is child's understanding of parent's mental illness?

6. Assessment of parent:

- Parent's ability to seek help
- Child's physical and mental development
- Impact of parent's disorder on child
- Parent's ability to meet child's needs
- Nature of parent's motivation and acceptance of responsibility
- Quality of support available
- Adequacy and effectiveness of current treatment
- Symptoms of mental illness, substance use and insight

HISTORY TAKING BEFORE STARTING OPIOID REPLACEMENT THERAPY

1. Does the patient have a diagnosis of opioid dependence?
2. Are there current signs of intoxication or withdrawal? Is there a risk for severe withdrawal?
3. Is the patient interested in opioid replacement treatment (ORT)?
4. Does the patient understand the risks and benefits of ORT?
5. Can the patient be expected to adhere to the treatment plan?
6. Is the patient willing and able to follow safety procedures?
7. Does the patient agree to treatment after a review of the options?
8. Can the needed resources for the patient be provided (either on- or offsite)?
9. Is the patient psychiatrically stable? Is the patient actively suicidal or homicidal; has he or she recently attempted suicide or homicide? Does the patient exhibit emotional, behavioral, or cognitive conditions that complicate treatment?
10. Is the patient pregnant?
11. Is the patient currently dependent on or abusing alcohol?
12. Is the patient currently dependent on benzodiazepines, barbiturates, or other sedative-hypnotics?
13. What is the patient's risk for continued use or continued problems? Does the patient have a history of multiple previous treatments or relapses, or is the patient at high risk for relapse to opioid use? Is the patient using other drugs?
14. Has the patient had prior adverse reactions to ORT?
15. Is the patient taking other medications that may interact with ORT?
16. Does the patient have medical problems that are contraindications to ORT?
17. Are there physical illnesses that complicate treatment?
18. What kind of recovery environment does the patient have? Are the patient's psychosocial circumstances sufficiently stable and supportive?
19. What is the patient's level of motivation? What stage of change characterises this patient?

FAMILY PLANNING AND OPIOID DEPENDENCE

6 months later, Rhonda is abstinent from heroin and wants to have another child with her partner. She is on a routine outpatient's review and wants to know your opinion about having a baby.

Outline the points will you cover during this interview?

- 1. Advise Rhonda that your role is not to offer relationship or family planning advice but to assist her in making an informed decision after explaining the risks and benefits.**
- 2. Therapeutic alliance:** Provide encouragement, validation and positive affirmation to Rhonda on her abstinence as part of motivational interviewing (maintenance phase)
- 3. Assess current mental state and social background:** look for evidence of mental illness that could result in relapse of substance use
- 4. Assess for craving and compulsion to use**
- 5. Assess psychosocial background :** relationship, finances, supports in the recovery environment
- 6. Inform Rhonda about the risks of opioid use in pregnancy**
- 7. Inform Rhonda about the peri natal issues in pregnancy and risk of relapse of mental illness**
- 8. Advice on involvement of specialist multidisciplinary drug and alcohol antenatal clinics.** Such a multidisciplinary team typically would include midwives, obstetricians, paediatricians, perinatal psychiatrists, social workers, child protection workers and addiction specialists.
- 9. Close liaison with obstetrics team and peri-natal mental health teams**
- 10. Choice of opioid in pregnancy in case of relapse would be methadone substitution therapy.**

11. Around delivery if on substitution therapy , admission would be preferable and post delivery monitoring of the baby due to the opioid withdrawal syndrome.

12. Provide written information and invite partner to have another discussion.

Key points :

Substitution treatment is the preferred approach for the opioid dependent pregnant woman due to its capacity to:

- improve access to antenatal care with improved birth outcomes
- reduce heroin and other drug use, and improve the health of pregnant women;
- reduce maternal and infant deaths associated with heroin use;
- reduce the spread of blood-borne communicable diseases associated with injecting drug use;
- facilitate the improvement in social functioning of the mother.

Although many women want to cease using opioids when they find out they are pregnant, opioid withdrawal is a high-risk option because a relapse to opioid use will expose both mother and child to the risks of unsanctioned drug use and affect the mother's capacity to care for the child. Opioid substitution treatment has better outcomes, both in access to antenatal care and ongoing stability, than withdrawal during pregnancy

Although there is no strong evidence, it is widely accepted that withdrawal is not advisable prior to 14 weeks, or after the 32nd week of pregnancy (K. Kaltenbach, Berghella, & Finnegan, 1998; A. R. Lingford-Hughes, et al., 2012), due to the risk of inducing spontaneous abortion or premature labour. Relapse to opioid use during pregnancy can also result in poor obstetric outcomes. Opioid substitution treatment is thought to have minimal long-term developmental impacts on children when compared to the risk of uncontrolled opioid use during pregnancy and resulting harms (WHO, 2009).

Methadone and buprenorphine can both be considered first line options for the management of opioid dependence during pregnancy. Both medications are safe and effective during pregnancy for both mother and neonate. (National Drug Strategy, 2014)

MEQ 11.1

A 41-year-old female presented to the emergency department found to be unconscious following an overdose. She has a past history of several presentations with deliberate self-harm. On medical work up it was found that she has atrial fibrillation too and was treated in a medical ward.

11.1. List the differential diagnoses

Delirium (In this question it is important not to jump to conclusions. A patient with a medical overlap should always have delirium ruled out)

1. The following are likely causes of delirium
 - Stroke
 - Pulmonary embolism
 - Haemorrhage including cerebral haemorrhage
 - Warfarin overdose (accidental or intentional)
 - Anti -arrhythmic overdose (prescribed for AF)
 - Drug and Alcohol Withdrawal or intoxication
1. Depressive disorder with suicidal ideation
2. Deliberate self harm as part of Borderline Personality Disorder
3. **You may add other causes that you think are likely but the key point is that despite a previous history of repeated self-harm one should still consider a hierarchical approach to evaluation.**

MEQ 11.2

Q 11.2 The medical team decides that she needs to be treated with warfarin and they want your advice. What is your advice to the medical team?

1. **Risk Benefit analysis:** Risk of Warfarin prescription to be weighed against risk of pulmonary embolism, stroke or death if warfarin is not prescribed.
2. **Risk assessment:** Suicidal ideation and Self harm ideation
3. Can newer anticoagulants (e.g Xarelto) be prescribed which carry a lesser risk and in a restricted supply?
4. If warfarin is to be prescribed then restricted supply to be considered with close INR monitoring
5. Case management
6. Possible daily prescribing by Assertive Outreach team

MEQ 11.3

Q 11.3. The lady is discharged back home with follow up in the community via a community care team. Devise a management plan for the next three months, which you will present to your consultant.

1. **Risk Management:** Current Suicidal and Self Harm ideation with current risk factors that can increase risk (e.g Substance Use, Depressive disorder, Lack of insight)
2. Mental State examination on Discharge
3. Appointments with team and frequency: regular appointments with follow up and MSE and risk assessment documented at each visit
4. Case manager to be appointed
5. Pharmacy involvement to provide dosette box and restricted supply. Doctor shopping line to be informed if present and informing pharmacies to avoid duplicate prescribing
6. Treatment of primary psychiatric disorder and co-morbidities
7. Inform family and family meeting if possible
8. Inform Chief psychiatrist of patient due to high risk of suicidality

MEQ 12.1

A 39-year-old school teacher was referred by her GP due to long term addiction to morphine tablets. Her husband has long term back pain. She has three children (15, 12 and 7). She confirms that she does not use them while at work but uses only two tablets at night-time. She has no anxiety or depression.

12.1. Describe your assessment

General Principles include the following: (Please adapt the answer according to the scenario)

1. **Risk Assessment** : parenting assessment to assess for risk of harm to children (Neglect or direct risk of harm). Indirect risk such as easy access to medication. Risk of misuse or overuse of opioids or other substances. Doctor shopping.
2. **Assessment of Pain**
 - Rating, duration, site, onset, duration, progress.
 - Treatment and effectiveness
 - Involvement of pain management specialist
 - Coping strategies
3. **Assessment of Substance Use Disorder (Refer notes)**
 - Substances used
 - Choice of drug
 - Onset, duration and progress
 - Amount and frequency
 - Features of dependence
 - Side effects (opioid side effects: nausea, sedation, constipation, cognitive deficits and confusion)
 - Tools such as Current Opioid Misuse Measure (COMM) and Opioid Risk Tool (ORT)
4. **Psychiatric history**
 - Depression, anxiety, mood disorders. Personality assessment to rule out dependent personality and other personality traits. Somatoform disorder.
 - Past history of Substance use

- Family history of mental illness or substance use
- Early childhood maladjustment
- Poor role models in early life
- Forensic history: involvement with police for drug or alcohol offences

5. Medical History

- DM, HT, medications prescribed, Epilepsy, Bowel dysfunction.
Medications that may have drug interactions.

6. Cognitive and Physical Examination:

- Do not forget examining for needle marks or stigmata of other substance use such as Alcohol. Pinpoint pupils.

7. Insight and Needs Assessment

- Motivational interviewing to determine where the patient is on the cycle of change (pre-contemplation to maintenance phase). Does she think the current use is problematic?
- Psychosocial needs: relationships (husband and children), vocation, accommodation and finances.
- Resilience and motivation to change along with psychological mindedness.

MEQ 12.2

12.2. Outline the key steps in management

This question was mainly asked to test understanding of Opioid Therapy Agreement (These principles apply to Benzodiazepine and Stimulant use)

1. Whilst dependence needs to be treated, it is important to recognise that abrupt stopping or forcing the patient to stop when they are not ready can lead to disengagement and diversion. (See Benzodiazepines in notes)
 2. Refer to a pain management specialist for optimum treatment of pain
 3. Motivational interviewing and building of therapeutic alliance
 4. Psychoeducation about the harmful effects of dependence to the patient
 5. A therapy agreement to be drawn up whilst patient continues to take medication prior to reduction and cessation
- Patients should use their prescribed medication only as directed by the doctor
 - They will be unable to secure replacement medication if the script is lost or stolen
 - They agree to receive the medication from only one doctor
 - They will not receive additional medication if the prescription runs out early
 - They will accept generic brands of medication
 - If function has not improved after a negotiated length of time then the doctor has the right to taper the medication
 - They agree to submit urine and blood samples and agree to participate in all aspects of therapy for pain and psychiatric disorder if present.
 - They agree to use only one pharmacy to fill their prescriptions
 - They are to be responsible in managing their own appointments

MEQ 12.2

12.2. Outline the key steps in management

This question was mainly asked to test understanding of Opioid Therapy Agreement (These principles apply to Benzodiazepine and Stimulant use)

1. Whilst dependence needs to be treated, it is important to recognise that abrupt stopping or forcing the patient to stop when they are not ready can lead to disengagement and diversion. (See Benzodiazepines in notes)
 2. Refer to a pain management specialist for optimum treatment of pain
 3. Motivational interviewing and building of therapeutic alliance
 4. Psychoeducation about the harmful effects of dependence to the patient
 5. A therapy agreement to be drawn up whilst patient continues to take medication prior to reduction and cessation
- Patients should use their prescribed medication only as directed by the doctor
 - They will be unable to secure replacement medication if the script is lost or stolen
 - They agree to receive the medication from only one doctor
 - They will not receive additional medication if the prescription runs out early
 - They will accept generic brands of medication
 - If function has not improved after a negotiated length of time then the doctor has the right to taper the medication
 - They agree to submit urine and blood samples and agree to participate in all aspects of therapy for pain and psychiatric disorder if present.
 - They agree to use only one pharmacy to fill their prescriptions
 - They are to be responsible in managing their own appointments

MEQ 13.1

A 35-year-old migrant lady from India and mother of a child who is 5 was admitted to the ED following a seizure in a supermarket. Her husband works far away from home and comes home only on weekends. He is unsupportive of her and she has minimal social support.

The emergency department assessment revealed that investigations are normal and medical registrar diagnoses this as a pseudoseizure and wants you to assess the lady.

13.1. Outline the approach to the assessment.

There are many ways of answering this but the key points to remember are:

1. **Do not assume that a pseudo-seizure diagnosis indicates absence of organic causes**
 2. **Cultural evaluation which includes cultural sensitivity**
 3. **Psychosocial issues predominate due to adjustment and losses secondary to migration.**
-
- **Risk Assessment:** Medical tests and examination to have medical causes ruled out. U and E (electrolyte disturbances), TFT's, MRI or CT to rule out head injury or intracranial causes, EEG to rule out true seizures, Blood glucose to rule out hypo or hyper glycaemia.
 - **Risk of harm to child:** Are DHS involved. Where is the child currently?
 - **Clarification of Seizure episode with medical registrar:** Onset, duration and progress. Presence of emotional stressors or trauma. Pseudoseizures have a gradual onset; presence of emotional disturbances, no micturition or defecation, lasts for more than half an hour and EEG is normal
 - **Psychiatric Evaluation:** Elements of cross cultural assessment (covered during course) and involve a multicultural health worker or translator if required. **The below is present in slides (issues for migrants).**
 - **Pre migration issues:** personality, childhood, early childhood adjustment issues etc.
 - **Migration issues:** reasons for migration etc.

- **Post migration issues:** Losses. Domestic violence, emotional and physical abuse. Features of depression, psychosis, mania and drug and alcohol misuse.
- **Past history of seizure and pseudo-seizures.**
- **Medication history:** Is she on anti epileptics?
- **Medical History:** Diabetes, Hypertension. Medication history: e.g. insulin
- **Psychosocial Evaluation:** Relationship stressors, Support with the child, Post natal depression after birth, Financial and accommodation stressors and Domestic violence
- **Cognitive and Physical Evaluation:** Physical examination to rule out bruises taking into account gender and cultural sensitivity e.g. get a female chaperone or female doctor. Cognitive impairment to rule out delirium and sequelae of seizures or pseudoseizures.
- **Family meeting** to assess for family dynamics and support available. Rule out stressors e.g. financial stressors
- **Cultural Aspects:** Abuse may not be disclosed unless a confidential and private setting is provided. Involve cultural liaison if necessary.

MEQ 13.2

13.2. Her husband calls angrily and says that she is acting out and does not have any medical grounds to be admitted. He wants her to be discharged home.

How would you respond to the situation?

This question is about foreseeability and knowing who to involve and why.

Risk Management:

- If risk of aggression is present, call security.
- Consider pacifying the husband and offer to have a meeting. Two people present and duress alarm in possession.
- **Psycho-education to husband** about current condition: nature and treatment (write content depending on the question and diagnosis for patient)
- **Speak to patient and clarify situation.** Ensure patient is free from undue influence in making decisions
- **Evaluate for domestic violence, physical or emotional abuse.** Involve a female worker in assessment if necessary. Involve social worker
- If evidence of domestic violence is present, police will need to be informed and DHS if risk to child is identified
- If ongoing medical risk is present the patient will need to be informed and treated under duty of care
- If mental health issues are present and the issues pose a significant risk of harm to self-harm or others then a mental health act assessment should be considered.

MEQ 14.1

A 75-year-old female who was suffering from Parkinsonism for 15 years was admitted to change the batteries of her pacemaker. On the second day after changing the batteries she had become disoriented, experiencing visual hallucinations. She lives with her 80-year-old husband and was managed by the GP. You are called by the medical team to see this lady. She was on lorazepam nocte on a long-term basis but the medical team stopped her lorazepam.

14.1. Describe your assessment of this lady with your differential diagnoses.

The key points are following

1. **Risk assessment:** Medical risk (physical illness), risk of harm to self, accidental self-injury or suicide and risk of harm to others. Where is the best place to evaluate and manage her?
 2. **Setting:** Consider safe setting e.g. 1: 1 special etc.
 3. **Collateral information to obtain past psychiatric and medical history.** State what you would like to know. E.g. medications, previous diagnosis, liaise with cardiologist
 4. **Evaluation to rule out (State the most salient features to rule out the following: clinical and investigations)**
 - Pacemaker syndrome
 - Delirium: Electrolyte disturbances, UTI, glucose abnormalities, Lorazepam withdrawal
 - Intracranial injury, stroke, PE, chest infection
 - Lewy Body Dementia
 - Psychosis with parkinsonism
 - Alcohol withdrawal
 - Epilepsy
-
1. **Cognitive and physical examination:** Delirium or Dementia, neurological examination to rule out stroke, rigidity, myoclonus etc.
 2. **Psychosocial Evaluation**

MEQ 14.2

A 75-year-old female who was suffering from Parkinsonism for 15 years was admitted to change the batteries of her pacemaker. On the second day after changing the batteries she had become disoriented, experiencing visual hallucinations. She lives with her 80-year-old husband and was managed by the GP. You are called by the medical team to see this lady. She was on lorazepam nocte on a long-term basis but the medical team stopped her lorazepam.

Describe your assessment of this lady with your differential diagnoses.

See above question (Here you should weigh on organic issues as visual hallucinations are organic unless proven otherwise and delirium should always be excluded first. Note that visual hallucinations in individual with Parkinsonism may indicate Lewy Body Dementia)

MEQ 14.3

14.3. Write down your medication advice for the medical team.

Main points

Reinstate lorazepam if this was ceased suddenly

1. Avoid antipsychotics or use extreme caution due to risk of severe rigidity, NMS or worsening of confusion.
2. If antipsychotics are to be used, Quetiapine small dose with regular monitoring of Pulse, BP, Temperature and physical examination to exclude NMS
3. Avoid anti-cholinergics as can worsen confusion

MEQ 14.4

14.4. The medical team decide to discharge her. What advice would you give to them before discharge?

1. Cognitive function: Is she delirious or is she stable?
2. Has a meeting with family or carer been arranged?
3. Occupational therapist to carry out functional assessment
4. Physiotherapist to check for mobility and home assessment
5. Assessment for Carer burden
6. Referral to psychogeriatric team for follow up
7. Case management
8. Medication and use of aids to enhance compliance and prevent confusion e.g. dosette box

MEQ 15.1

A young man is brought to the emergency department on a hunger strike. He is physically debilitated when presented. He is in a cubicle with two other people in it. You are asked to see this patient?

Q 15.1. List the ethical considerations in this case.

Ethical considerations

1. Duty to act ethically.

Doctors must try to prevent coercion or maltreatment of detainees and must protest if it occurs.

2. Respect for autonomy.

Doctors should respect individuals' autonomy. This can involve difficult assessments, as hunger strikers' true wishes may not be as clear as they appear. Any decisions lack moral force if made involuntarily by use of threats, peer pressure or coercion. Hunger strikers should not be forcibly given treatment they refuse. Forced feeding contrary to an informed and voluntary refusal is unjustifiable. Artificial feeding with the hunger striker's explicit or implied consent is ethically acceptable.

3. 'Benefit' and 'harm'.

Physicians must exercise their skills and knowledge to benefit those they treat. This is the concept of 'beneficence', which is complemented by that of 'non-maleficence' or *primum non nocere*. These two concepts need to be in balance. 'Benefit' includes respecting individuals' wishes as well as promoting their welfare. Avoiding 'harm' means not only minimising damage to health but also not forcing treatment upon competent people nor coercing them to stop fasting. Beneficence does not necessarily involve prolonging life at all costs, irrespective of other values.

4. Dual Agency Dilemma. (This is particularly relevant when issues with detainees occur)

Acting in the patient's best interest vs. Loyalty to the employing authority (e.g. Government). Clinical independence: Physicians must remain objective in their

assessments and not allow third parties to influence their medical judgment. They must not allow themselves to be pressured to breach ethical principles, such as intervening medically for non-clinical reasons.

5. Confidentiality

The duty of confidentiality is important in building trust but it is not absolute. It can be overridden if non-disclosure seriously harms others. As with other patients, hunger strikers' confidentiality should be respected unless they agree to disclosure or unless information sharing is necessary to prevent serious harm. If individuals agree, their relatives and legal advisers should be kept informed of the situation.

6. Gaining trust.

Fostering trust between physicians and hunger strikers is often the key to achieving a resolution that both respects the rights of the hunger strikers and minimises harm to them. Gaining trust can create opportunities to resolve difficult situations. Trust is dependent upon physicians providing accurate advice and being frank with hunger strikers about the limitations of what they can and cannot do, including where they cannot guarantee confidentiality.

MEQ 15.2

15.2. Patient was given feeds and was medically stabilised after several weeks. The medical team inform you that they are going to discharge him.

What are the key points you would like to assess before discharging him?

This question is based on the following: The question is at discharge stage so should be adapted accordingly but the general principles will help.

GUIDELINES FOR THE MANAGEMENT OF HUNGER STRIKERS

1. Capacity to make decisions:

This involves verifying that an individual intending to fast does not have a mental impairment that would seriously undermine the person's ability to make health care decisions. Individuals with seriously impaired mental capacity cannot be considered to be hunger strikers. They need to be given treatment for their mental health problems rather than allowed to fast in a manner that risks their health.

- 1. As early as possible, physicians should acquire a detailed and accurate medical history of the person who is intending to fast.** The medical implications of any existing conditions should be explained to the individual. Physicians should verify that hunger strikers understand the potential health consequences of fasting and forewarn them in plain language of the disadvantages. Physicians should also explain how damage to health could be minimised or delayed by, for example, increasing fluid intake. Since the person's decisions regarding a hunger strike can be momentous, ensuring full patient understanding of the medical consequences of fasting is critical. Consistent with best practices for informed consent in health care, the physician should ensure that the patient understands the information conveyed by asking the patient to repeat back what they understand.
- 2. A thorough examination of the hunger striker should be made at the start of the fast.** Management of future symptoms, including those unconnected to the fast, should be discussed with hunger strikers. Also, the person's values and wishes regarding medical treatment in the event of a prolonged fast should be noted.

3. Sometimes hunger strikers accept an intravenous saline solution transfusion or other forms of medical treatment. **A refusal to accept certain interventions must not prejudice any other aspect of the medical care, such as treatment of infections or of pain.**
4. **Physicians should talk to hunger strikers in privacy and out of earshot of all other people, including other detainees.** Clear communication is essential and, where necessary, interpreters unconnected to the detaining authorities should be available and they too must respect confidentiality.
5. **Physicians need to satisfy themselves that food or treatment refusal is the individual's voluntary choice.** Hunger strikers should be protected from coercion. Physicians can often help to achieve this and should be aware that coercion may come from the peer group, the authorities or others, such as family members. Physicians or other health care personnel may not apply undue pressure of any sort on the hunger striker to suspend the strike. Treatment or care of the hunger striker must not be conditional upon suspension of the hunger strike.
6. If a physician is unable for reasons of conscience to abide by a hunger striker's refusal of treatment or artificial feeding, the physician should make this clear at the outset and refer the hunger striker to another physician who is willing to abide by the hunger striker's refusal.
7. Continuing communication between physician and hunger strikers is critical. **Physicians should ascertain on a daily basis whether individuals wish to continue a hunger strike and what they want to be done when they are no longer able to communicate meaningfully.** These findings must be appropriately recorded.
8. When a physician takes over the case, the hunger striker may have already lost mental capacity so that there is no opportunity to discuss the individual's wishes regarding medical intervention to preserve life. Consideration needs to be given to any advance instructions made by the hunger striker. **Advance refusals of treatment demand respect if they reflect the voluntary wish of the individual when competent.** In custodial settings, the possibility of advance instructions having been made under pressure needs to be considered. Where physicians have serious doubts about the individual's intention, any instructions must be treated with great caution. If well informed and voluntarily made, however, advance instructions can only generally be overridden if they become invalid because the situation in which the decision was made has changed radically since the individual lost competence.
9. **If no discussion with the individual is possible and no advance instructions exist, physicians have to act in what they judge to be the person's best interests.** This means considering the hunger strikers' previously expressed wishes, their personal and cultural

values as well as their physical health. In the absence of any evidence of hunger strikers' former wishes, physicians should decide whether or not to provide feeding, without interference from third parties.

10. **Physicians may consider it justifiable to go against advance instructions refusing treatment because, for example, the refusal is thought to have been made under duress.** If, after resuscitation and having regained their mental faculties, hunger strikers continue to reiterate their intention to fast, that decision should be respected. It is ethical to allow a determined hunger striker to die in dignity rather than submit that person to repeated interventions against his or her will.
11. **Artificial feeding can be ethically appropriate if competent hunger strikers agree to it.** It can also be acceptable if incompetent individuals have left no unpressured advance instructions refusing it.
12. Forcible feeding is never ethically acceptable. Even if intended to benefit, feeding accompanied by threats, coercion, force or use of physical restraints is a form of inhuman and degrading treatment. Equally unacceptable is the forced feeding of some detainees in order to intimidate or coerce other hunger strikers to stop fasting.

Other practical considerations:

1. Medical and Psychiatric evaluation before discharge
2. Follow up: Case management
3. Social worker to address issues of socio-political nature: Legal status
4. Case conference to address **dual agency dilemmag**. patient may not have Medicare and hence may not receive care even though from an ethical point of view, one has to act in the best interest of the patient
5. Collaborative Approach: NGO's welfare groups, family involvement

Reference:

1. WMA Declaration of Malta on Hunger Strikers

MEQ 16.1

A 32-year old Caucasian female who lives with three children is referred to you for sleep disturbance. She says she has been diagnosed with chronic fatigue syndrome and is preoccupied with aches and pains in her joints. She says she is constantly tired during the day.

16.1. Describe your assessment

Key points: All basic principles of approach discussed during course apply (e.g. risk, information gathering etc.)

1. **Exclude Psychiatric causes of fatigue:** Depression, Somatic Symptoms Disorder, Anxiety Disorder, Substance use (including benzodiazepine's) Personality factors (this is important and emphasis should be given as the personality traits can often maintain the disorder)
2. **Rule out Sleep disorder**
3. **Physical and Medical Causes of fatigue** through physician involvement and investigations: Hypothyroidism, Addison's disease, Iron deficiency Anaemia, UTI , Connective Tissue disease, Fibromyalgia etc.
4. **Chronic Fatigue Syndrome:**

Fatigue with following features:

New or recent onset

Persistent or recurrent

Unexplained by other conditions

Reduction in activity

Post exertion malaise and fatigue

AND one or more of following

Difficulty sleeping

Muscle and joint pain (multi site)

Headaches

Painful lymph nodes

Sore throat

Cognitive dysfunction

Flu like symptoms

1. **Medication history:** pain medications, benzodiazepine treatment (often patients have dependence on substances)
2. **Cognitive and Physical exam:** to exclude issues above
3. **Psychosocial history:**

Early childhood maladjustment

Role models with pain or abnormal illness behavior

Learned behavior i.e. physical pain as care eliciting behavior

Secondary gain: current stressors

Stressors with children and lack of psychosocial support

MEQ 16.2

You diagnose her with depression. She reports that she had a family history of bipolar affective disorder in her family. Her mother also had thyroid dysfunction. Growing up she was prone to infections and often spent several months in hospital for severe urinary tract infections. **She missed school as a result**. Her parents were critical of her and favoured her brother. Her mother was addicted to benzodiazepines and she described her mother as emotionally distant. She performed poorly at school and was below average. She was bullied at school for being overweight. She tells you that currently her pain and fatigue prevents her from going to work and that the only thing that gives her relief is Lorazepam.

16.2. Outline the key contributory factors to this patient's depression providing justification from the vignette.

Such questions are about developing a Formulation: each factor mentioned has a link to the disorder based on your understanding of predisposing, precipitating and maintaining factors

1. **Family History: Genetic Predisposition** towards an affective illness
2. **Family history of Thyroid dysfunction:** Thyroid dysfunction can be a direct **biological predisposing factor** in fatigue and depression
3. **Repeated infections** may be a sign of **poor immunological function** that can predispose her to later life fatigue
4. **Spending several months in hospital** could have led to separation from care giver at early age leading to **attachment difficulties** which can lead to low self esteem through dysfunctional assumptions of self e.g. Being different from other children, not able to develop peer relationships (**Cognitive Model**)
5. **Missing school:** Impacting social skills and development of peer group relationships (**Eriksonian model**)
6. **Parental criticality:** Development of low self-esteem and internalization of harsh critical parents setting up vulnerability for depression in later life (**Cognitive and Psychodynamic Model**)
7. **Mother addicted to Benzodiazepines:** Role modeling of abnormal illness behavior i.e. maladaptive coping strategies (**Abnormal illness behavior and Sick role**)
8. **Emotionally distant mother:** Alexithymia in family leading to decreased expression of emotions in family, which is a predisposing

factor in somatoform illness and depression. It would also lead to attachment difficulties. **(Attachment model)**

9. **Poor school performance:** Biologically it may represent a neurodevelopmental issue. Psychosocially would have impacted on self-esteem.
10. **Bullied at school for being overweight:** Reinforcement of low self-esteem leading to core schema of self doubt, self loathing all of which are a predisposing factor for depression
11. **Pain and fatigue prevent her from going to work:** This may be related to cognitive distortions about pain and function along a **cognitive- behavioral model. Secondary gain may be present (Sick Role).**
12. **Lorazepam use:** Cognitive distortion about lorazepam being the only medication that gives her relief. It may also indicate maladaptive coping strategies as a learned behavior from childhood

MEQ 17.1

A 25-year-old Caucasian female who is a known patient of the mental health team is 24 weeks pregnant and presents to you in crisis as she has an upcoming court case for custody of her 3-year-old child.

17.1. List the key principles you would consider in management.

Key points

Risk assessment

Risk to self, unborn child and risk to 3-year-old child (Direct and indirect risk)

Involvements of Child Protection Services to assess further risks and explain to patient reason for involvement

Psychosocial support

1. **Attempt to engage the patient** and build a therapeutic alliance.
Explain that involvement of Mental health services is to support her and her family and allay her fears that being mentally unwell will lead to her losing her children.
2. **Setting of management:**

Low threshold for admission in a safe and secure

environment if risks are identified with care arranged for younger child.
Involve social worker.

1. **Specialist service involvement:** Perinatal Mental health service
2. **Antenatal Care**
3. **Case Conference to discuss management:** GP, Psychiatric Team, Specialist services, Child Protection if involved, Social Worker, Psychologist, Obstetrics team
4. Discuss with supervisor
5. **Liaison with court through court reports** (if requested) after taking dual agency dilemma into account. Explain the dual agency dilemma.
6. **Psychiatric assessment** for psychiatric disorders
7. **Parenting assessment**

MEQ 18.1

A Vietnam Veteran aged 67 presents to the emergency department late at night. The nursing staff call you to assess him as he has a ligature mark around his neck. On assessment he tells you that he was “mucking about” and that he had no intention of dying. He smells of alcohol.

18.1. List the key points in management

Candidates are able to answer this question based on principles we discussed during course but some not to be missed points are as follows :

1. **Risk assessment:** The situation should be taken seriously and a low threshold for admission should be considered due to discrepancy between the severity of attempt and the comment about “mucking about”.
2. **Mental Health act vs. Duty of care.** Management of risk if wanting to abscond (e.g. one on one special / security).
3. **Rule out Alcohol related disorders:** Intoxication, withdrawal and cognitive impairment. So a medical evaluation is important and administer thiamine early.
4. **Assessment of PTSD and Co-morbidity:** Depression, PTSD and Alcohol dependence are highly co-morbid. Patients can minimise the disorders hence a collateral in this case is very important.
5. **Adopt a dual diagnosis approach in management**
6. **Recognise the barriers in management,** which are likely to be an elderly male who may be stigmatised by mental health and therefore may not engage. So motivational interviewing, therapeutic alliance should be highlighted.

MEQ 19.1

You are a consultant psychiatrist in a community mental health team. The GP has referred Mandy, a 35-year-old Caucasian female with depressive symptoms for diagnostic clarification. Mandy's brother died 3 months ago in a car crash.

19.1. Outline the key differential diagnoses you would consider to assist the GP.

This question was posed mainly to focus on the difference between Grief reaction and Depression

Important points are as follows

1. In grief predominant affect is of emptiness and loss
In depression, the predominant affect is a persistent depressed mood and inability to anticipate happiness or pleasure

1. The dysphoria in grief is likely to decrease in intensity over days and weeks and occurs in waves called pangs of grief. (These waves are associated with thoughts and reminders of deceased)
2. Thought content of grief is preoccupation with thoughts and memories of the deceased rather than self-critical or pessimistic ruminations seen in major depressive disorder.
3. In grief self-esteem is preserved, in Major depression self-loathing and worthlessness is common.
4. In grief the individual thinks about death and dying, such thoughts are focused on deceased and possibly joining the deceased. In Major Depression, suicidal thoughts are present because of feeling worthless and undeserving of life

MEQ 20.1

You are a Consultant Psychiatrist in the community mental health team. You are seeing Mr Smith a 34-year-old junior medical officer who has been referred by his director of training for psychiatric evaluation. The director of training tells you that the medical officer has appeared drowsy during the morning shifts and suspects addiction.

20.1. Outline your approach to management.

Key principles

Clarify your role: Role of a psychiatrist or colleague? You are responsible for patient safety and hence notification may be necessary irrespective of your role.

1. **Professional, legal and ethical obligations:** Statutory Obligations that need to be followed.
2. **Duty to warn**, especially if there is a risk to the patient. The primary concern is safety of the patient. If safety is a concern then doctor should be advised to take leave immediately.
3. **Maintaining privacy and confidentiality** of the doctor and the limits that are imposed on this.
4. **Clinical Governance** issues that include corporate risk.
(Documentation and referral)

6. Management of the situation, which includes local interventions, external help and advice through professional bodies and support from the Registration Board.

1. In the case of a Psychiatrist who suspects an impaired doctor, he should first discuss this with his line manager.
2. The line manager then assesses the situation, gathers evidence and passes that evidence on to the Director of Clinical Governance and further on to the Chief Executives.
3. It is the Chief Executive's responsibility to then make appropriate notifications to Registration Authority. (AHPRA)
4. As mentioned earlier, it is important to keep the doctors under question, informed at all times and advise to seek legal advice, advice from his Medical Defence Union and Indemnity insurers as well.

All registered health practitioners (including psychiatrists) and employers have a legal obligation to notify AHPRA if they form a reasonable belief that another health practitioner has engaged in notifiable conduct.

5. Health notifications must be made in writing and usually those providing notification to the Board must be aware that the doctor who is the subject of notification must, as a matter of natural justice, be given a copy of the notification and any additional material that is provided.

The following are the principles in managing a doctor with drug and alcohol issues:

Establishing dialogue

- **Who?** It is usually better to have more than one person present when raising the issue of the problem of substance abuse. This helps to reinforce the gravity of the situation, allowing colleagues to show they are united and their wish to tackle the issue and help the impaired practitioner.
- **How?** The impaired doctor is likely to feel defensive and ganged up on. Accordingly, concerns should be raised with sensitivity and in a non-judgmental manner but an expression of concern.
- **Why?** It is better for the impaired doctor to know that the issue is in the open with colleagues and that they are united in their wish to tackle it and to help him/her.
- **When?** The issue should be raised when the impaired doctor is not intoxicated and as soon as practicable after the event that has led to the suspicion of impairment.
- **Where?** This should be done in a quiet and private place where interruptions are unlikely.
- **What?** State the facts focusing on work performance (What happened, when it happened, who was involved). Do not assume anything about the cause, drug use may be one possible cause but the primary issue is work performance. Express concern about the doctor as well as patient safety. Anticipate anger, denial, alternative explanations and expression of competence.
- **Psychological assessment based on role: The following issues should be covered.**

Personal causes:

- Blues (depression)
- Booze (alcohol)
- Bi-lingual (language)
- Babies (stressors)
- Bank (financial stressors)

- Birds/Blokes (relationship issues)

External or supervisor causes.

- Lack of support
- Lack of debriefing
- Lack of orientation
- No appraisal or feedback in place
- Exams
- Bullying or harassment
- Unsupportive registrar
- Nature of the work

System causes

- Poor rostering
- Long hours
- Stressful job
- Funding issues within the service

ASSESSMENT OF ATTACHMENT

Exam Question:

Mary is a 36 year old married woman. Three months ago she gave birth to twins via emergency caesarean section, born at 26 weeks. Her son Dennis died. Her daughter Kym is in neonatal ICU on a respiratory ventilator. Mary has remained in hospital since the birth. Nursing staff say Mary is frequently tearful and criticises their care of Kym. Nursing staff have begun avoiding being assigned to Kym because of this.

Her husband James is an accountant who works long hours and visits for around an hour at a time. Her parents live near by but say they are reluctant to visit as they “cannot stand seeing the baby like that”.

- Discuss HOW you would assess Mary’s attachment to Kym (Below is a generic attachment assessment. This can be adapted to the question. It is also suited if a parenting capacity attachment is asked)

1. Background:

Mary Ainsworth differentiated styles of attachment in infants. This research was based on the infants’ responses to separation from and reunion with their mother in an unfamiliar laboratory situation, known as the ‘Strange Situation’ (Ainsworth, Blehar, Waters & Wall, 1978).

The Strange Situation involved studying the communication between mother and child following 3 minute contrived separations. As a result of this research, Ainsworth grouped the patterns of attachment between infants and their carers into 3 main groups: ‘secure’, ‘insecure-avoidant’ or ‘insecure-ambivalent’.

2. Assessment of parents:

- **Any history of or current mental illness?** : Children with depressed mothers are more likely to develop insecure attachments (Rutter, p. 558)
- **Have parents experienced any significant losses?** : Children of parents with a history of loss and/or trauma are more inclined to form disorganised attachments

- **Is there substance abuse?** : Unpredictable or unreliable caregiving can also create attachment difficulties for children
- **Is there a history of or current domestic violence?** : Children exposed to domestic violence are 4 times more likely to have attachment disorders
- **What is the parent's capacity to reflect on the child's experience?** : Is it reality based? Sensitive? Flexible/accommodating? How does parent act on separation from the child?
- **How does parent act on reunion with the child?**: Does she respond appropriately, in a timely manner?

3. Assessment of child (Depending on age):

- **Any significant separations/disruptions from parents/primary carers?** : Circumstances of the separation/disruption Why?
- **What was the quality of care received from primary carer?** : For how long?
Who cared for the child?
- **What is the child's experience of care?**: According to carers, child, family members, professionals, your own observations
- **Model of care**
- Stable, reliable, unreliable?
What are the child's expectations of being looked after? How does the child get his/her needs met?
How does the carer respond to the child?
- **How old is the child?**
- **What stage of development is he/she at?** : < 6 months, 6-12 months, 1-2 years, 2-4 years, etc. Any developmental difficulties?
- **What is the nature of the child's interaction with primary carer?**
- **How does child relate to other adults?**
- **How does child relate to other children?**
- **How does child respond when separated from carer?** Before, during, after (Refer to styles of attachment in diagram)
- **How does child respond when reunited with carer?** Before, during, after (Refer to styles of attachment in diagram)

4. Who are the significant others in the child's life?

5. Has the child been in an out of home placement?

Reference: DHS Victoria

ANTIDEPRESSANTS AND BREAST FEEDING

Q. You diagnose Mary with post-natal depression. She is expressing breast milk to bottle-feed, and is keen to continue this. Describe the content of your discussion with Mary and her husband regarding the use of antidepressants and breast feeding.

1. Risk benefit analysis to help the couple make an informed decision
2. Clarify concerns first
2. Breast feeding confers many advantages to mother and baby and is encouraged (e.g promotes bonding, enhances child immunity)
3. Impact of postnatal depression on mother and baby : Post natal depression can affect bonding, cognitive and emotional development of the baby. Therefore treatment of depression takes priority
4. Antidepressants that are safe during breast feeding are available and many have low passage into the breast milk. (Motherrisk)
5. Most antidepressants are excreted in low concentrations in breast milk, with few reaching 10% of the maternal weight-adjusted dose. Paroxetine and sertraline produce low relative infant doses in the 0.5% to 3% range, while fluoxetine, venlafaxine, and citalopram produce milk levels closer to, and sometimes even above, the 10% limit
6. Alternative treatments for depression include CBT but may take time to act.
7. Provide leaflets to read about risk
8. Arrange another appointment to clarify concerns.

STAFF MANAGEMENT OF POSTNATAL DEPRESSION

Q. Nursing staff report Mary is difficult and ask your advise on how to handle her.

Discuss your response to this request

1. Listen and acknowledge difficulty in managing patient
2. Provide positive affirmation and appreciation of their role
3. Provide psycho education about postnatal depression: Symptoms include irritability, anger with psychodynamics mechanisms of acting out, projection and displacement. She would be particularly concerned about her young child who is in ICU.
4. Environmental techniques: Calming environment, speak softly and listen, attempt to establish rapport, try to involve familiar staff and involve family
5. Psychological techniques: Provide supportive counselling, CBT, relaxation and mindfulness techniques
6. Medication only if situation escalates and mental state worsens or if patient requests it after risk benefit analysis.

LITHIUM AND PREGNANCY

A 35 year old female with an established diagnosis of Bipolar Disorder is on Lithium 900 mg note for the last 2 years. She has been stable with no relapses over the last 2 years. She attends your outpatient clinic and tells you that she has just come from her GP's rooms who sent her here as she is now 8 weeks pregnant. You have not seen her before.

Outline your approach to the situation.

1. Attitude to pregnancy: If she is wishing to continue or terminate. If wishing to terminate, assess mental state and capacity to do so.

2. Risk-benefit analysis : Risk of stopping Lithium vs Risk of continuing Lithium treatment

3. Obtain background history from patient and reading notes: Severity of relapses, number of admissions, risk associated during relapses (suicide and harm to others).

4. Explain to patient that your role will be explain all risks and benefits to help her make an informed decision.

5. Offer to meet with husband or speak to husband at same time via teleconference. Important to involve her partner.

6. Explain background risk of congenital malformations in pregnancy. 2-4% (NICE guidelines)

7. Explain risk of Lithium exposure in first trimester.

First trimester use of lithium is associated with an increase in cardiac defects, particularly Ebstein's Anomaly which is increased from 1: 10-20,000 to 1: 1000. For this reason a high resolution ultrasound (foetal echocardiogram) will be carried out at 6 and 18 weeks.

8. Stopping Lithium can pose a risk of relapse which can impact on the ability to self care and make decisions in the interest of the baby and self. Relapse can increase risk of harm to self and others. High chance of relapse which can be as high of 70% when stopped before conception.

9. Close liaison with obstetrician, antenatal nurses and prenatal mental health team. Regular fetal ultrasounds

10. Provide leaflets with risks and benefits. Baseline risk and Absolute and relative risks of Lithium.

11. If mother wants to stop the Lithium then gradual discontinuation with close follow up to detect relapse.

12. Other aspects to take into account in pregnancy are:

a. Changed renal clearance in pregnancy may require increasing the Lithium dose as pregnancy progresses. Lithium levels, renal and thyroid function need to be monitored regularly throughout the pregnancy.

b. Due to risk of toxicity with rapid shift in maternal fluid volumes at pregnancy, the Lithium dose needs to be decreased 24-48 hours prior to a planned delivery, or ceased at the onset of spontaneous labour and recommenced at pre pregnancy levels after delivery.

DEPRESSION, ECT AND MEMORY COMPLAINTS

A 39 year old female with a 2 year history of depression has recently had a course of ECT (6 months ago). Over the last month she is complaining of worsening memory and slowing of her mind.

Outline the key features in assessment that you would focus on to evaluate the memory complaints

1. Temporality of ECT and memory complaints : Did memory complaints start immediately after ECT or were they present prior?
2. Temporality of depression and memory problems. Were memory complaints present prior to mood issues?
3. Onset of first memory issues and mood issues
4. Type of ECT given: Bitemporal or Bifrontal or Right Unilateral. Were there any complications
5. Psychiatric history: Previous memory complaints during mood episodes, Alcohol or substance use disorder, medication misuse (benzodiazepines, opioids etc.). Antipsychotics or sedative antidepressants or mood stabilisers. History of ADHD.
6. Cognitive examination: Memory loss with ECT is usually associated with retrograde and anterograde amnesia, autobiographical memory loss and loss of cognitive functions such as learning and attention) See here. Disorientation in Time, Place and Person indicates an organic possibility. Frontal lobe examination : Luria three step test, FAB etc. Other standardised test such as NUCOG.
7. Medical history: Thyroid disease, Vitamin deficiencies, Iron deficiency anaemia, High Blood pressure which can be associated with vascular neurocognitive impairment, family history of neurodegenerative disease
8. Physical examination: Neurological examination: Cogwheel or lead pipe rigidity, tremor, bradykinesia (parkinsonism), Romberg's sign, motor weakness for evidence of TIA's.

9. MRI scan and /or EEG done previously

10. Investigations: To rule out medical disorders listed above.

GERIATRIC PSYCHIATRY- ASSESSMENT PRINCIPLES

Mr Alan Woods is a 64-year old male who is brought to the CMHT by his daughter due to a change in personality. His daughter reports that he has been doing odd things and has said inappropriate things to his grandchildren.

Question:

Describe your assessment with respect to establishing the diagnosis?

1. Risk assessment :

- a. Risk of harm to self : suicidal ideation / self harm ideation and accidental self harm
- b. Risk of others: aggression
- c. Risk to children: Clarify inappropriate things said. Is there evidence of physical or sexual abuse? Child protection may need to be involved if evidence of abuse is present.
- d. Risk of driving, wandering , risk to reputation

2. Background history : Who does he live with? Is he dependant in activities of daily living? What supports does he have? Who looks after finances, cooking, cleaning etc.

3. Clarify situation through functional analysis : Onset, duration and progress of behaviour. If acute, may indicate medical cause. Antecedents, behaviours and consequences.

4. Medical history and medical evaluation: History of hypertension or diabetes for vascular dementia. Rule out Urinary tract infection, Thyroid dysfunction, electrolyte disturbance, medication history e.g. diuretics, anti-hypertensives can be associated with delirium.

5. Evidence of delirium : sudden onset, reversal of sleep wake cycle, hallucinations or delusions, fluctuating cognition.

6. Psychiatric History: Does he have a previous history of depression, mania or psychosis. Is he on medication?

- Rule out mania: sleep disturbance, excessive spending, euphoria, pressure of speech and tangentiality
- Has there been a personality change, apathy, disinhibition which can indicate cortical major neuro-cognitive impairment. Word finding difficulty can indicate Alzheimer's dementia. Sub-cortical dementia can be ruled out by presence of cog wheel rigidity, tremor, gait disturbance, primitive reflexes. Disinhibition usually indicates orbito-frontal lobe involvement. Lewy body dementia is differentiated by spontaneous features of parkinsonism and visual hallucinations
- Assess for **behavioural and psychological symptoms of dementia** (BPSD) : wandering, incontinence, insomnia, aggression, sexual disinhibition.
- Substance use disorder: Alcohol use history which can present with intoxication or dementia from long standing use.

7. Cognitive examination to rule out fronto-temporal dementia which can present with disinhibition. Tests include FAB, Luria-3 step, NUCOG, Clock Drawing test. MRI and SPECT to rule out dementia i.e atrophy of frontal or temporal lobes.

8. Physical examination: neurological examination to rule out focal neurological deficit, rigidity, ataxia indicating subcortical involvement . Evidence of vascular disease, cardiovascular examination. Alcohol stigmata.

9. Family history of neurodegenerative disease such as dementia, parkinsonism, early stroke, personality change and psychiatric disorder.

10. Psychosocial environment: Finances (pension, capacity to make financial decisions, Enduring power of attorney), Relationships (assess for carer burden), Accommodation (functional capacity with a functional assessment after acute situation has resolved). Capacity to make medical decisions for self.

GERIATRIC PSYCHIATRY - GENETIC TESTING PRINCIPLES

You diagnose him with Alzheimer's dementia. His daughter tells you that she has read that dementia can be hereditary and wants to know if her family should be tested.

What issues would you discuss?

1. Acknowledge the sensitivity of the situation and provide support and clarify any questions she may have.

2. Provide psychoeducation about general pros and cons of genetic testing

- Genetic testing creates opportunities for the assessment of risk and diagnosis but there are important limitations. Not all genetic tests provide the information that families want and therefore can lead to uncertainties. e.g having a particular gene does not indicate the severity or the expression of the illness
- Some conditions are not treatable therefore the question arises if family members should be tested for an untreatable disease
- No test can detect a diagnosis with 100% sensitivity or specificity which means there are people that will incorrectly diagnosed as positive or negative.
- In some cases, genetic testing may limit the persons access to insurance.
- Genetic testing may help with forward planning.
- Most of the risks associated with genetic testing involve possible emotional, social or financial consequences of the test results. You may feel sad, angry, frightened, depressed, anxious or guilty after learning your results. Learning that one carries a mutation in a gene that results in a progressive, lethal disorder can be devastating and should be considered very carefully.

On the other hand, test results can give you a sense of relief from uncertainty and help you make informed decisions about managing your health care. The test results might impact life decisions, such as career choice, family planning, or insurance coverage. Your genetic counselor can explain in detail the benefits, risks and limitations of a particular test. It is important that you understand and weigh these factors before making a decision.

3. Specific information regarding Genetics of Alzheimer's Disease.

Alzheimer's disease is divided into Early onset and Late onset Alzheimer's disease. Early onset Alzheimer's disease occurs in people aged 30-60. It is rare and accounts for less than 5% of all people that have Alzheimer's disease. Most cases of Early onset are inherited and therefore is known as Familial Alzheimer's disease. FAD is caused by mutations on genes 21, 14 and 1. A child whose mother or father carries a mutation for FAD has a 50-50 chance of inheriting that mutation. If the mutation is inherited the child will almost surely develop FAD.

Late onset Alzheimer's occurs after 60 (her father). There is no one gene associated with this late form of disease. There are certain genes that can increase the risk of the disease and one such gene is the Apoe4 but having the gene does not mean one will surely develop the disease. A blood test exists to identify which Apoe allele a person has but it does not predict who will or will not develop the disease.

4. If patient wants more specific advice refer to a **genetic counsellor**

5. Provide leaflets to read and arrange another meeting if necessary and involve other family members.

Huntington's Disease

The mutant gene is situated on Chromosome 4 and in the autosomal dominant mode of inheritance, each offspring of an affected individual has a 50% chance of inheriting the mutant gene. If they inherit the gene they will surely develop huntington's disease.

Fronto-temporal dementia:

Mutations are found in 30–40% of cases with a positive family history. There are at least five genes associated with FTD and its subtypes. The microtubule associated protein tau gene (MAPT) was the first FTD gene identified in 1998 followed by the progranulin gene (GRN) in 2006. The MAPT and GRN mutations are variably found in some 10–20% of familial FTD series. FTDP-17 showing genetic linkage to chromosome 17, presents with a clinical syndrome of autosomal dominant disinhibition, dementia, parkinsonism, and amyotrophy.

Approximately 20-50% of individuals with frontotemporal dementia (FTD) have an affected first-degree-relative. Conversely, 50-80% of individuals

appear to be the first person with FTD in the family, also called sporadic or nonfamilial FTD. Among individuals with FTD, approximately 10% have a change in a single gene (also called a mutation). Single gene causes for FTD are inherited in an autosomal dominant manner, meaning each child of an affected parent has a 50% chance of inheriting the change and also developing the condition. Currently, changes in five genes have been associated with autosomal dominant FTD. It is possible additional genes will be identified in the future. Therefore, not finding a change in one of these genes does not reduce the risk for family members to zero. At this time, changes in the following five genes have been identified:

- MAPT gene on chromosome 17 that makes the protein tau,
- GRN gene, also called the PGRN gene, on chromosome 17 that makes progranulin protein,
- TARDBP gene on chromosome 1 that produces trans-acting response DNA-binding protein of 43-kDa molecular weight (TDP-43),
- VCP gene on chromosome 9 that codes for valosin-containing protein and
- CHMP2B gene on chromosome 3 that expresses charged multivesicular body protein 2B (also known as chromatin modifying protein 2B).

Mutations in the MAPT and GRN genes on chromosome 17 are the most common genetic causes of FTD. Clinical genetic testing for MAPT, GRN and VCP is available. Genetic testing is usually coordinated through a genetic counselor or other genetics professional following a genetic counseling appointment and detailed three-generation pedigree. Avoid jargon when writing in exam and explain what you would tell the patient.

CAPACITY ASSESSMENT - CONSULTATION-LIAISON PSYCHIATRY

A few months later Mr Woods has a fracture of his femur and is admitted to the surgical unit. After the operation, Mr Woods has developed gangrene of his foot. He refuses to have this operated. The surgeons call you to speak to Mr Woods.

How would you manage the situation?

1. **Speak to the surgical team and clarify situation:** What surgery is required, what are alternatives, seriousness and severity of the situation.

2. **Therapeutic alliance:** Speak to Mr woods on his own and attempt to build a therapeutic alliance. Adopt a non-judgemental approach. Explain your role as a consultation-liaison doctor as patient may not want to talk to a psychiatrist

3. Explain the role of a capacity assessment to the patient

4. **Ethical dilemma:** Autonomy of the patient vs. Duty of care.

5. Components of capacity assessment :

- Does Mr Woods understand the purpose of the surgery and the consequences of not having surgery? Does he understand the risks and benefits of the surgery?
- Does Mr Woods understand the alternatives and the risks and benefits of alternatives?
- Can Mr Woods weigh up the pros and cons of the situations and arrive at a balanced decision and what is the quality of the decision.
- Can Mr Woods communicate the decision appropriately?
- Is Mr Woods free from undue influence? e.g family
- Is the decision consistent?

6. If Mr Woods has capacity and refuses to have surgery then liaise with surgical team and provide ongoing support for Mr Woods. Sometimes the patient may change their decision as the situation deteriorates. Obtain advice from guardianship tribunal and obtain a second opinion.

7. Provide audio-visual aids, involve family and consider religious and spiritual beliefs.

8. Treat any mental illness present. In the context of mental illness if there is a risk of harm to self due to impaired capacity to make decisions then mental illness can be treated under the MHA.

9. If he lacks capacity, then arrange case conference with surgical team and obtain second opinion. Speak to Guardianship tribunal as best practice. Proceed in the best interest of patient and with the least restrictive option. Take into account wishes of family and any advance directives.

IMAGING IN DEMENTIA

This was a previous exam question

Typical neuroimaging findings in the major types of dementia

Standard axial: generalised cerebral atrophy and ventricular enlargement Angled axial: reduced medial temporal lobe width	Temporo-parietal hypoperfusion
Medial temporal lobe atrophy Periventricular white matter lesions (seen as areas of hypoattenuation on CT and as hyperintensities on proton density and T2-weighted MRI scans)	
Infarct/s Extensive deep white matter lesions	Patchy multi-focal pattern of hypoperfusion
Generalised ventricular enlargement	Posterior deficits (often similar to Alzheimer's disease)
Relative preservation of medial temporal disease lobe structures	Reduced D2-receptor density and dopamine transporter
Similar white matter changes to Alzheimer's disease but less extensive than vascular dementia	
Frontal lobe atrophy	Anterior perfusion deficits

INTERPERSONAL SOCIAL RHYTHM THERAPY

Interpersonal Social Rhythm therapy is an evidence based treatment for bipolar disorder and is based on the relationship between life events and bipolar disorder taking into account the social zeitgeber theory. Zeitgeber is an external or environmental cue that affects the biological or circadian rhythms.

The articulation of the social zeitgeber theory led to the development and testing of IPSRT (Frank, 2005), which integrates IPT for unipolar depression with a behavioural intervention that focuses on social rhythm and sleep/wake regulation. In IPSRT, the clinician helps the patient recognise the relationship between disruptions in social rhythms and the onset of previous episodes and then uses standard behavioural techniques, including psychoeducation, self-monitoring, successive approximation, and reinforcement (via the observed link between regular routines and improved mood), to facilitate and maintain increases in social rhythm and sleep/wake regularity.

IPSRT addresses the interplay between the inter-personal and biological spheres by helping patients to see how interpersonal stressors and social role transitions can disrupt daily routines important to maintaining circadian integrity. As in IPT, therapists aid patients in resolving interpersonal disputes and negotiating role transitions. At the same time, they teach patients how to regularise irregular daily routines and how to maintain such regularity even in the face of external challenges.

Key principles:

- 1) the reciprocal relationships between life stress
- 2) and the onset of mood disorder symptoms, the importance of maintaining regular daily rhythms and sleep-wake cycles, and
- (3) the identification and management of potential precipitants of rhythm dysregulation, with special attention to interpersonal triggers. Therapists instruct patients to monitor their social routines and rhythms using the Social Rhythm Metric and, ideally, to begin to see the interplay among instabilities in daily routines, patterns of social stimulation, sleep/wake times, and mood fluctuations. IPSRT then implements behavioural strategies to help patients alter activities that promote rhythm irregularities (minimising

overstimulation, monitoring the frequency and intensity of social interactions). Patients are encouraged to make life changes in order to protect the integrity of their circadian rhythms and sleep/wake cycles.

INTELLECTUAL DISABILITY : APPROACH TO ASSESSMENT

John is a 22-year-old male with **Down's syndrome** who lives in a **residential home** for people with **learning difficulties**. You are a psychiatric registrar working in the emergency department. The ED registrar refers John's case to you after John was **verbally and physically aggressive to staff in the residential home**. The **staff refuse to have him at the home until his behaviour has settled**.

Question

6

Outline your approach to the assessment.

Note similarities with Geriatric Psychiatry Assessment in Neurocognitive impairment. This answer focuses on the main principles so apply it to the patient in the question to make it non-generic.

1.Ensure John's safety priory to assessment : 1 on 1 special to avoid risk of harm to self or others and absconding.

2. Speak to staff and clarify situation: Onset, duration and progress of behaviour. Was this a single incident or a pattern is evident? Functional Analysis (Antecedents, Behaviours and Consequences). Identify the key worker with whom John has known the longest or has best therapeutic relationship.

3. Assessment of baseline environment and psychosocial functioning of the patient: Number of residents, male-female ratio, number of staff, 24 hour residential or Day time support, medication supervision, Shift schedule, usual activities in the group home, age group of patients.

4. Assessment of communication skills of patient (Pre-symbolic, Symbolic or verbal), Cognitive and social functioning: Does he cook , clean , self hygiene or is he dependant on others for activities of daily living?

5. Preparation in advance before seeing patient : Informant rated interview schedule such as Mini PAS-ADD ahead of interview with main carer. Allow longer time with patient, avoid interruptions, use pictures, leaflets and audio-visual aids such as Books beyond Words (Hollins, 2010).

6. Approach to interview with patient : Closed questioning may be problematic as they may give responses that the questioner is expecting or may repeat the last choice. Open questioning may also cause problems as they require the person to think and retain a number of possible answers whilst at the same time having to structure their response in a certain way.

Questions should be broken down into small chunks, and check understanding. Use simple, short sentences with everyday words and avoid jargon. Use pictures, photographs, facial expressions and tone of voice to facilitate communication. Give time to formulate a response and allow for mild inappropriateness and be flexible.

7. Assess for Causes of behavioural disturbance:

a. Medical Causes: Constipation, Urinary tract infection, ear ache, respiratory infections. dental pain, headache (Menstruation in females), headache. Onset of early Alzheimer's in Down's. Thyroid dysfunction and epilepsy. Electrolyte abnormalities.

(b. Behavioural phenotypes: Does the patient have a diagnosis of Prader Willi Syndrome/ Lesch Nyhan or Fragile X syndrome) if the diagnosis is not given. Here it is Down's syndrome.

c. Environmental causes: Distress, change in routine, change in carer or residents, bereavement

d. Behaviour serving a function (secondary reinforcement) : To gain attention from carers, to avoid chores or activities, to obtain tangibles, to avoid boredom.

e. Psychiatric Causes: The diagnosis of mental illness in individuals with intellectual disability is challenging because the presentations occur atypically. Therefore misdiagnosis can occur and is known as diagnostic overshadowing. Longitudinal observation in a safe place may be necessary to note diagnosis. Change in sleep wake cycle, fluctuating cognition may indicate delirium, suspiciousness or attention to invisible stimuli may indicate psychosis or epilepsy related syndromes, tearfulness or withdrawal may indicate depression and disinhibited behaviour may indicate affective syndromes. Decline in functioning may indicate neurocognitive impairment and a scan may be required.

INTELLECTUAL DISABILITY : MANAGEMENT PRINCIPLES IN BEHAVIOURAL DISTURBANCE

John's behaviour settled and he went to the residential home. The staff ask for particular strategies to manage John's behaviour when he gets aggressive.

Outline the key points would you cover in your discussion?

1. **Recognise and acknowledge difficulty** in management and be sensitive to the carer. Provide positive affirmation and offer support.
2. **Clear advice on risk management:** If significant risk of harm to the patient or others then crisis team or police may need to be informed first. Provide appropriate numbers.
3. **Advice on functional analysis** and maintaining a diary to note, antecedents, behaviours, consequences with times. This can help detect a pattern.
4. **Practical strategies for staff about interviewing techniques:** See previous question.

Make individual as comfortable as possible, identify familiar caregiver, encourage use of comforting objects, quiet spot, use gestures, soft voice, warmth, simple words and avoid jargon.

5. **Advice on causes of behavioural disturbance :** previous question

6. **Advice on management of behavioural disturbance:**

- a. Environmental techniques : de-escalation, time out, one on one supervision
- b. Psychological techniques: functional analysis, Using positive reinforcement, avoid punishment
- c. Pharmacological: low dose antipsychotics and benzodiazepines taking into account susceptibility to side effects

7. **Support for care giver:** respite, staffing issues and ability to raise concerns.

INTELLECTUAL DISABILITY - RELATIONSHIPS AND ETHICAL DILEMMAS

John has developed a relationship with a 24 year old female in the nursing home? The staff are concerned and so are John's parents. The staff want to discuss this with you.

What are the main issues to be addressed in this situation?

1. Ethical issue: Autonomy and privacy of the patient vs. duty to protect vulnerable individuals. Consumers have a right to express sexual freedom and is a key tenet of the recovery model. It is important at the same time that consumers are not exploited or harmed in anyway because they are unable to make appropriate decisions.

2. Refer to nursing home policy and speak to clinical director.

3. Speak to the mental health team of the 24 yr old female and advise of the situation.

4. Independent capacity assessments to be carried out by the respective teams.

5. Sexual education to provided to both parties independently so that they are able to make informed decisions; Seek advice from local sexual health teams. : Education about anatomy and biology, education about what constitutes a normal relationship and sexual act. Family planning and contraception. Use audio visual aids, pictures, books and check understanding. Avoid jargon.

6. Assess capacity to consent to relationship (Both teams separately) :

- What is the nature of the relationship?
- Why does he or she want to have a relationship.
- Does the person know it is wrong to force someone to have sex?
- Does the person understand that the extent of a sexual relationship is guided by mutual enjoyment?
- How does the person consent and what is the nature of these choices?
- Is there a power imbalance in the relationship?
- Is consent free from undue influence and is there consistency in the decision making?

- Is the person able to weigh the pros and cons of the decision?

7. **Arrange a meeting with family** with clinical director and explain the issue after obtaining consent from the patient.

8. **Seek legal advice** if the patient lacks capacity e.g guardianship bodies, medico legal insurance.

ASSESSMENT OF RISK - STALKING

Jane is a 35 year old nurse working in an in patient unit where you work as the psychiatric registrar. Steven is a 35 year old male who presents to the reception of the in patient unit demanding to see Jane. He claims that he is in love with her and wants to see her immediately. He seems agitated. Jane reports to you that she does not know him and he has been following her lately. She mentions that he has been an inpatient on the unit 3 months ago.

Question: **Describe your assessment and what are the key features you would like to focus on with a view to diagnosis**

1. Risk management: Inform security. Ensure safety of staff . Duress alarm. Ensure adequate means of escape present. Inform NUM. Security to search for weapons. Attempt to establish rapport whilst being cautious not to aggravate the situation. Inform police depending on level of threat. It is often safer to call for help early and to remain at a safe distance until support, such as police and ambulance, arrives. A show of support from other agencies may persuade the patient to cooperate.

2. Approach to patient and management of aggression or agitation: (The detail depends on the number of marks. This can be asked as a separate question. Divide the management into environmental, medication and psychological techniques)

- Use an empathic non-confrontational approach, but set boundaries
- Listen to the patient, but avoid giving opinions on issues and grievances beyond your control
- Offer food, drink and a place to sit
- Avoid excessive stimulation
- Avoid aggressive postures and prolonged eye contact
- Recruit family, friends, case managers to help
- Address medical issues especially pain and discomfort
- Try to ascertain what the patient actually wants and the level of urgency
- Offer oral medication
- In severe cases of agitation or aggression, the mental health act may need to be used. **(While there are state by state differences in the Mental Health Act the principles are very similar. When involuntary care is needed, an initial schedule is written to allow**

safe care and transport to a mental health unit (this may be an emergency department, psychiatric unit or hospital). It should be remembered that in some jurisdictions a Mental Health Schedule can now be written by police and ambulance officers as well as by a doctor, and that it is a legal order that a patient be taken to a place where they can be assessed by a mental health specialist. If there is a potentially serious medical emergency it may be necessary to provide treatment without immediate scheduling of an uncooperative patient. Restraint and forced sedation should be considered a last resort.)

- IM medication and seclusion may be needed depending on level of threat. Monitor patient's physical state (T,P, RR , O2 sats after IM injections)

2. Review previous notes to assess for diagnosis and risk: Does he have a psychotic disorder and are there any forensic convictions or risk of harm to self or others previously? Does he have a case manager. Is he on an involuntary order. Medications such as oral or depot medications? Past aggressive behaviour, severity, use/possession of weapons , Past threats, recent threats. Substance abuse, Past impulsivity, Poor psychosocial supports, Recent severe stress, loss, threat of loss, Past indications/diagnosis of antisocial personality disorder

3. Assess mental state: Evidence of delirium , organic causes (hypoglycaemia, substance withdrawal, head injury, seizure activity etc.) Evidence of psychosis , substance misuse , mania , erotomanic features.

- Intoxicated
- Cognitively impaired
- Psychotic and/or manic
- Delusions or hallucinations focussed on a particular person • Command hallucinations
- Preoccupation with violent fantasy
- Delusions of control (especially with violent themes)
- Agitation, excitement, hostile, suspicious
- Frustrated, angry
- Pacing, yelling, not cooperating
- Medical – pain, acutely unwell

4. Establish stalking behaviour :

- Unwanted contacts, including following, approaches and surveillance

- Unwanted communications, including telephone calls, e-mails, letters and graffiti
- **5. Associated risk with stalking :**
- Threats
- Assault
- Ordering or cancelling goods or services on the victim's behalf
- Sending unsolicited gifts
- Initiating spurious legal actions
- Making vexatious complaints
- **6. Risk assessment related to stalking :** Use relevant components of the HCR-20.
- **7. Establish type of stalker:**
- The rejected who pursue ex-intimates, either in the hope of reconciliation, or for vengeance, or out of a mixture of both
- Intimacy seekers who stalk someone they believe they love and who they think will reciprocate
- Incompetent suitors who inappropriately intrude on someone, usually seeking a date or brief sexual encounter
- The resentful who pursue victims to exact revenge for some actual or perceived injury
- The predatory whose stalking forms part of sexual offending
- **8. Duty to warn third party** e.g nurse of risk of harm is evident.

RISK FACTORS FOR VIOLENCE OR AGGRESSION

The HCR-20 can be used OR the following

Static Factors:

- Age
- Gender
- Ethnicity
- Intellectual disability
- Social class
- Educational level
- Developmental trauma
- Criminal history
- History of previous dangerousness

Dynamic and Stable

- Treatment non- responsiveness
- Personality
- Interpersonal support network
- Impulsivity
- Problem solving capacity
- Cognitive schema
- Attribution style
- Attachment style
- Immature or primitive psychological defences

Dynamic and Changeable

- Psychotic symptoms
- Mood or anxiety symptoms
- Substance use
- Acute crisis
- Social isolation
- Unsatisfactory living situation
- Treatment non-adherence

As you can see you can expand on some of the above to get more factors.

KEY POINTS IN SEVERE AGGRESSIVE BEHAVIOUR

- These disturbances are the most severe, characterised by actual physical aggression or assaultive behaviour.
- Such patients are often severely distressed by psychotic symptoms or disinhibited by alcohol or other drugs. These instances represent psychiatric emergencies and warrant management including use of physical restraint, seclusion, the administration of parenteral psychotropic medication and the appropriate level of monitoring.
- The safe restraint of such patients, to protect both staff and the patient from injury, requires special training.
- Psychotropic medication administered parenterally, particularly when benzodiazepines are combined with antipsychotic medications, may be unpredictable in their effects.
- Seemingly small doses can lead to catastrophic complications such as upper airway collapse or respiratory arrest. As such, parenteral psychotropic treatment should only be used when adequate resuscitation facilities are available.
- The patient must be monitored for signs of cardiovascular instability, oxygen desaturation and airway protection.
- The circumstances of seclusion or administration of emergency parenteral antipsychotic medication require careful documentation in both the patient's chart and a formal 'seclusion register'.

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STALKING - MANAGEMENT OF VICTIM

Jane the inpatient nurse has been 'traumatised' by the overall experience and comes to you for advice. She reports feeling constantly on edge and believes this is affecting her work. She tells you that she has developed a dislike for young male patients on the ward.

List the key issues in management

1. Clarify your role (Professional): Are you the treating doctor or a colleague? Explain the if there are performance issues that may impact on patient care then you have an obligation to report.

2. Support for a colleague (Manager): Be supportive, express empathy, provide advice on avenues for Jane to obtain support : OHS, NUM, Supervisor, insurance etc. Work place has an obligation to provide a safe and supportive place for the employee.

3. Assess for any risk of harm to patients : Is the nurse unable to perform due to her situation and is this impacting on patient care? If this is the case then inform NUM and Clinical Director as reporting obligations to regulatory body would be important.

4. Independent psychiatric assessment to rule out PTSD, Adjustment disorder or onset of depression that may impact on her work to be arranged by work place.

5. Referral for supportive counselling, psychology and time off.

6. Counselling to address psychodynamic issues of transference and countertransference developed through the traumatic experience.

7. Support for Jane: There is an obligation for work place to act in the best interest of the employee.

- Safe and confidential therapeutic setting
- Education and supportive counselling
- Cognitive-behavioural therapies for anxiety and avoidance
- Pharmacotherapy for psychiatric and physical morbidity
- Substance misuse counselling

- Family and partner counselling
- Support groups
- Support organisations

Further reading

Stalking and Assessment of stalkers

PARAPHILIAS - ASSESSMENT

Principles in evaluation:

1. Demographic characteristics:

age, gender, current and past marital status, number of children (age and gender if any), current and past employment status (with or without children), education level;

2. Clinical Characteristics:

- conventional and paraphiliac sexual fantasies and activity (frequency and type), exclusive or non exclusive paraphiliac behaviour, age at onset of paraphiliac behaviour, type and number of paraphilia, gender and age of victims, intra familial or not (known or unknown victim), internet use or video use, violence, previous convictions for sexual or non-sexual offences, family and personal history of sexual disorders, previous treatments.
- for sexual offending and compliance, alcohol or illicit drug consumption before paraphiliac sexual behaviour, age of puberty.

3. Risk factors:

- Personal history of psychiatric disorders, suicide attempts, history of brain trauma, current dementia, previous or current psychiatric or non-psychiatric diseases and treatment, previous history of sexual abuse or use of violence
- Family history of paraphilias, drug or alcohol misuse and psychiatric disorders.

4. Stage of Change and readiness for treatment:

- victim empathy, coping with stress, impulsivity, interpersonal relationships, insight, motivation for treatment, cognitive distortions, denial, degree of mental retardation if any.

5. Diagnosis of paraphilia.

Number and type of paraphilias; comorbidity with axis 1 or axis 2 of the DSM classification (especially addictive disorders or personality disorders); comorbidity with somatic diseases if any; cognitive evaluation if mental

retardation or dementia; careful medical examination, blood measurements and/or plasma hormone levels if hormonal treatment is needed. Baseline osteodensitometry could be necessary in case of hormonal treatment.

PARAPHILIAS : TREATMENT

Psychological Treatment:

1. Cognitive-behavioral therapy (CBT)

Applying behavioral therapy techniques to modify sexual deviations by altering patients' distorted thinking patterns and making them cognizant of the irrational justifications that lead to their undesirable sexual behaviors. It may be employed in accordance with a 7-step approach, as follows:

- Aversive conditioning with ammonia or (masturbatory) satiation
- Confrontation of cognitive distortions (especially effective in groups)
- Victim empathy (showing videos of victims and the consequences they experience from the patient's act)
- Assertiveness training (including social skills training, time management, and structuring)
- Relapse prevention (identifying antecedents to the behavior [high-risk situations] and ways of disrupting these antecedents)
- Surveillance systems (family associates who help monitor patient behaviour)
- Lifelong maintenance

One can also use sexual impulse control and biofeedback along with above.

2. Orgasmic reconditioning: By using masturbation, recondition the patient to orgasm to a more acceptable stimulus.

3. Social Skills Training : Sexual education, developing intimacy, carrying on conversations with others, and assertive skills training.

4. Group therapy and psychodynamic therapy.

Pharmacological Treatment:

- Antidepressants, such as lithium and various selective serotonin reuptake inhibitors (SSRIs)
- Long-acting gonadotropin-releasing hormones (ie, medical castration), such as leuprolide acetate and triptorelin
- Antiandrogens (to lower sex drive), such as medroxyprogesterone acetate (10 mg q12hr, with the dosage doubled every 3 days to a

maximum of 200 mg/day, then maintained for 1 month and adjusted as necessary)

- Phenothiazines, such as fluphenazine
- Mood stabilisers

Surgical Treatment: Bilateral orchidectomy

Ethical issues arise in surgical and medication treatment.

The major ethical issues regarding sex offenders including paraphilias may reflect the need for public safety balanced against the public and even professional orientation towards punishment rather than treatment even where treatment is appropriate and effective.

PATHOLOGICAL GAMBLING - ASSESSMENT

1. Detailed assessment of gambling behaviour:

- initiation
- progression
- current frequency (days per week or hours per day)
- current severity (money spent on gambling proportionate to income)
- types of games played
- maintaining factors
- features of dependence

2. Diagnosis of Pathological Gambling:

- **Needs to gamble** with increasing amounts of money in order to achieve the desired excitement.
- **Is restless or irritable** when attempting to cut down or stop gambling.
- Has made **repeated unsuccessful efforts** to control, cut back, or stop gambling.
- Is often **preoccupied with gambling** (e.g., having persistent thoughts of reliving past gambling experiences, handicapping or planning the next venture, thinking of ways to get money with which to gamble).
- Often **gambles when feeling distressed**.
- After losing money gambling, often returns another day to get even (**"chasing" one's losses**).
- Lies to conceal the extent of involvement with gambling.
- Has jeopardised or lost a significant relationship, job, or educational or career opportunity because of gambling.
- Relies on others to provide money to relieve desperate financial situations caused by gambling.

3. Consequences: financial, interpersonal, vocational, social and legal

4. Reasons for consultation, motivation to change and expectations of treatment

5. Assessment of suicide risk

6. Assessment of Axis I and II comorbidity, including substance use disorders

7. Comprehensive mental state examination to rule out mania, depression, OCD

8. Questionnaire's :

- The South Oaks Gambling Screen (SOGS; Lesieur & Blume, 1987)
- The Lie/Bet Questionnaire (Johnson et al, 1997)
- Gamblers Anonymous's Twenty Questions (the GA-20; Gamblers Anonymous, 2005)

General Points:

Aetiological models in gambling

- Psychoanalytic (unconscious desire to lose, unresolved Oedipal conflicts)
- Learning theories (monetary gain and excitement acting as positive reinforcers)
- Cognitive theories (cognitive distortions such as magnification of one's gambling skills, superstitious beliefs, interpretive biases)
- Neurotransmitter theories (serotonin, noradrenaline and dopamine dysfunction).

PATHOLOGICAL GAMBLING - MANAGEMENT

1. Pharmacological Treatment:

a. SSRI : all SSRI's have shown to be better than placebo

b. Naltrexone:

μ-opioid receptor antagonist, is effective in the treatment of a range of impulsive behaviours/disorders such as kleptomania, self-injurious behaviours and borderline personality disorder. It is also useful in reducing high-urge and craving states in people dependent on alcohol and heroin. Naltrexone's predominant mechanism of action is via the modulation of the mesolimbic dopamine pathway involved in reward and reinforcement.

c. Mood stabilisers particularly Lithium in patients with and without bipolar disorder

2. Psychological Therapy:

a. Behavioural therapy: Aversion therapy, imaginal desensitisation, imaginal relaxation, behavioural monitoring, covert sensitisation and spousal contingency contracting.

b. Cognitive Behavioural Therapy:

Other treatments often incorporated in cognitive-behavioural packages include training in assertiveness, problem-solving, social skills, relapse prevention and relaxation.

c. Gamblers Anonymous

ASSESSMENT OF ATTEMPTED SUICIDE AND DELIBERATE SELF HARM

Assessment of attempted suicide and DSH

1. Build Rapport

A patient who is being seen following self-harm or for the assessment of suicidal intent may be distressed, embarrassed or guarded and, therefore, maybe reluctant to engage or cooperate with history taking. However, patients are often relieved by the unburdening of their troubles rather than being annoyed or offended.

2. Psychiatric History

Approach to the history : Open and direct manner without ambiguity so that mistakes are not made. Introduce questions regarding suicide in a sequential manner. For example, starting with “With all these problems that you are now facing, have you ever thought that you would rather be dead?” If an affirmative answer is given then it could be asked “Have you ever thought about deliberately ending your life?”, then “Have you thought about how you might do this?” It is often useful to run through a chronological description of the events leading up to, during and after the self-harm or suicide attempt to assess the level of risk.

Key points to cover :

a) prior to self-harm:

- Significant acute psychosocial stressors (possible precipitating factors) or medical problems
- Low mood or symptoms of a major mental illness
- Feelings of being better off dead
- Feelings of hopelessness
- Drug or alcohol consumption
- Preparation for death; finalising will or life insurance, giving away possessions, writing a suicide note
- Onset of suicidal ideation
- Degree of planning (versus impulsivity)
- The patient’s perception of the degree of lethality of the chosen means patient may strongly believe that five sleeping tablets would be lethal in

overdose) and the patient's intent (e.g. to die, to escape problems, to sleep)

b) events at the time of the suicidal act:

- The setting; where they at home with family around them or did they attempt suicide away from home or at a time when they knew no one would be around
- Was the patient intoxicated?
- Acute stress present? (e.g. argument with partner)

c) post self-harm:

- Are they glad or disappointed that they are alive?
- Does the patient show remorse or regret about the attempt? (Shame or regret may be a good or bad thing: some patients will regret the hurt that loved ones may have felt and be less inclined to attempt suicide in the future; a person with low self-esteem may feel even more of a burden or worthless and therefore more determined to carry out further attempts)
- Actions or behaviour after self-harm or suicide attempt (e.g. immediately called someone for help or tried to hide the attempt from others)
- Continued access to suicidal means (e.g. does the security guard who presents after self-harm have access to a gun?)
- Willingness to engage with mental health services and accept treatment
- Ongoing or future suicidal intent or plans
- Plans for the future? Do they express plans to see friends, keep appointments or to try to obtain goals in the future?
- What supports are available in the community?
- Has the self-harm served a purpose or helped out the patient in someway (e.g. release of frustration, mobilised the support of loved-ones) that may result in a reduction of risk?
- If the patient denies further suicidal intent or plan following an attempt, what has changed? (e.g. an acute stressor has passed, they have come to a realisation that they are loved or that their death would be more significantly felt by others than they previously thought, social supports generated etc.) If there doesn't appear to be any change in the patient's situation following a serious suicide attempt but the patient is denying further suicidal intent then consideration should be given to whether the patient is being deliberately deceptive.
- Look for discrepancies in the recall of events that may indicate that the patient is being deceptive (e.g. a patient may present after an 'accidental' injury or overdose in the context of significant psychosocial

stressors and have significant risk factors for suicide but denies suicidal ideation or intent) Consider suicidal intent in a patient that has been involved in uncharacteristic risk-taking behaviour; a patient presents obtunded after taking a large amount of benzodiazepines but upon awakening say that their intent was just to get some sleep.

- The ability of the patient to guarantee their safety is not a reliable measure of risk.

3. Past psychiatric history:

- Ask about previous suicide attempts; the psychosocial context in which they occurred, the method used, degree of intent and lethality and treatment sought or provided
- Patient's ability to engage with treating professionals or teams
- Presence or absence of diagnosed mental illness or personality disorder

4. Collateral History: Obtain collateral history from past medical files, family, friends, general practitioner, treating psychiatrist or psychologist, or community mental health team

Issues of privacy and confidentiality must be weighed against the level of risk; if the patient does not give consent to talk to third parties, then confidentiality may be broken if it is felt that the level of risk to the patient (or others) outweighs the patient's right to privacy; a judgement may also need to be made about whether there is enough concern to risk jeopardising the therapeutic alliance from contradicting the patient's wishes.

5. Medical/ Physical Assessment:

- Consider a paracetamol level and other drug screen after self-harm as a routine (not all patients are reliable historians), if the medications ingested are unknown or if there has been a suspected polypharmacy overdose
- Consider a longer period of medical observation for medications with unusual metabolism or those that are slow release
- Patients who are intoxicated with alcohol or other substances may need to be detained in hospital and observed until they are sober so that a more thorough risk assessment can be undertaken
- Assessment of cognitive function may be important as part of an assessment capacity if the patient is requesting to leave or to detect ongoing cognitive side effects of ingested substances.

MANAGEMENT PRINCIPLES - ATTEMPTED SELF HARM AND SUICIDE

1. Risk management:

- **Sedative medication may be required to reduce distress or reduce risk of harmful behaviour**
- **1:1 nurse special, security, police depending on level of threat.**
- It is important that a patient is medically stable prior to being transferred to a psychiatric ward.

2. Treatment Setting:

1. Does the patient need to be admitted or could treatment be provided in the community?

This will depend on the patient's need for medical management, their degree of risk, level of support in the community and their willingness to engage in treatment.

2. If the patient is to be admitted should this be as a voluntary patient or under the Mental Health Act?

- Depends on the degree of risk and the patients level of cooperation with treatment. Use the least environment.

3. When a patient is admitted consideration will need to be given to the type of ward and the level of nursing care

- If the patient requires close nursing supervision or is at risk of absconding then it would be appropriate that patient is managed in a closed ward, or observation or high dependency unit
- For patients considered to be of high immediate risk of self-harm consideration should be given to 1:1 nursing care
- There may be a lower threshold for 1:1 nursing care on a medical or surgical ward as the expertise of the nursing staff to provide psychiatric care will be low.

4. Psychiatric Management/ Modification of Risk Factors:

- Reduce psychological distress or symptoms.
- Increase social support
- Offering alternatives to suicide (e.g. through problem solving techniques)

- Treat underlying psychiatric illness or substance abuse/dependence.

CAUSES OF DELIRIUM IN ELDERLY

MISTE – a mnemonic for possible causes of delirium

M : Metabolic – hyponatraemia, hypoglycaemia, hypoxaemia

I : Infective – urinary tract infection, pneumonia

S : Structural – subarachnoid haemorrhage, urinary retention

T : Toxic – drugs (e.g. digoxin, lithium) or poisons, alcohol

E : Environmental – being in hospital or the emergency department

Reference: Managing delirium in older patients, Aust Prescr 2011;34:16-8

PSYCHIATRIC ASSESSMENT OF ABORIGINAL PATIENTS - PRINCIPLES

1. Build rapport:

- Be introduced to the patient by a familiar staff member
- greet the patient with a loose handshake and brief eye contact
- ensure adequate personal space, and give an explanation of who you are and of your role.
- Ernest Hunter advocates starting the interview with a genogram. This helps to quickly establish family relationships, losses, and living arrangements and conveys interest in the patient as a person. It also places the patient in the position of expert.
- Use non-threatening statements (such as commenting on events within a community or the person's life) to put people at ease.
- Create a problem list with the patient. This focuses the interview on the patient's priorities. Use humour, especially at your own expense.
- Keep language simple and clear. Make the patient a cup of tea, a gesture which symbolises hospitality, humility, freeness with time.

2. Communication

- Informed listening: demonstrate an understanding about salient background matters that put the patient's story in a context. Listen to both silences and what is said.
- Use open-ended questions. Indigenous people may feel confronted by direct questioning and give any answer, whether correct or not, in order to deflect attention. It is more important to make a connection with the patient, so talk about things other than the mental health issue first, talk around topics, and accept that not all information may be gathered in one sitting, but that investment in the relationship is most important.
- Talk slowly and wait patiently for a response (quick responses are seen as impolite), and be aware that anxiety about the interview may affect behaviour.
- Particular cultural considerations include not referring to a dead person by name, taboos associated with the use of personal names, recognising that spiritual experiences are not necessarily psychotic, observing cultural norms (e.g. brief or intermittent eye contact, sit beside rather than opposite the patient), checking relationships and sense of belonging to country and family, awareness of the significance of spiritual issues and recognition of the effect of gender of the interviewer (if opposite to the patient, they may feel uncomfortable and unable to disclose information) and transference issues.

- Obtain corroborative information from family, Aboriginal health workers and other involved clinicians and members of the community.
- Concerns about confidentiality must be weighed up, but obtaining accurate information is important. Clarify which family members are significant and address confidentiality and consent to give information early in the process.

3. Mental state examination

Appearance, behaviour, rapport: Establishing the patient's usual level of self care may help distinguish what is pathological (for example hygiene, the state of clothing) from normal grooming; recognising that scars may be the result of traditional rituals and not self-harm; appreciating that shyness is common and so avoidance of eye contact may not represent illness.

Speech: Responses may be delayed, softly spoken and short. The clinician must also consider the patient's familiarity with English.

Mood: If mood is not volunteered, then offering suggestions with words commonly understood in the local community (e.g. "wild" for anger, "silly" for euphoric, "weak" for depressed and "strong" for good or well).

Affect: Crying is uncommon as many Aboriginal children brought up traditionally are taught not to cry as it may cause sickness. Shyness and shame may also be mistaken for a flat or depressed affect. A patient may appear blank or expressionless with the clinician and yet be animated and reactive with relatives or familiar people.

Thought form: Disturbances of thought form may be more difficult to detect if the patient is not fluent in English. Seek the opinion of relatives, Aboriginal health workers and liaison officers.

Thought content: Interpreting the clinical significance of thought content requires awareness of accepted cultural beliefs. Check with the Aboriginal liaison officer or other clinician. For example, in some communities in far North Queensland black magic may be considered a cause of sickness or death and it may be accepted that the spirits of the deceased move around the living and are perceptible at times.

Perceptual abnormalities: Fleeting visual hallucinations such as spirits may be reported in the context of intense emotional experiences. However, auditory hallucinations are more likely to indicate mental illness.

Cognition: Be aware of biases which may adversely affect performance on Western psychological tests. Check knowledge of familiar material (e.g. sporting teams), observe behaviour in the community (assesses performance in everyday tasks), and check their cognitive reputation (talk to people close to the patient).

Time: Aboriginal people often place events in a circular, rather than linear, pattern of time. Events are placed in time according to their relative importance for the individual, with more important events located as 'closer in time'. Assess event/time orientation using culturally and personally relevant events such as 'memory milestones' (e.g. seasons, deaths, and family gatherings). Assess cortical function by having the patient name common objects, copy a drawing of 2 intersecting boomerangs, Luria hand sequences and primitive reflexes.

Insight and judgement: Take into account cultural beliefs and norms, including traditional explanations of illness.

References:

Sheldon, M., (2001). Psychiatric assessment in remote Aboriginal communities. Australian and New Zealand Journal of Psychiatry, 35: 435-442.

Hunter, E., (2004). Communication and Indigenous health. Australian Doctor, 4 June: 35-42.

CHILDHOOD ADHD ASSESSMENT

You are a psychiatry registrar working in CAMHS. Jack is a 10 year old boy who lives with his mother Lucy aged 32 and his father 37. He has a younger sister Mary who is 6 years of age. The GP has referred Jack to your service as he has been 'very difficult to handle'. His teachers are complaining that he is disruptive and his academic performance is deteriorating.

Outline your approach to the assessment of Jack.

1. Gather information before the appointment : Parent and Teacher Strengths and difficulties questionnaire and Conner's Questionnaire to obtain information about behaviours and impact of behaviours. Clarify if there is any risk of harm to self or others as this may require more urgent intervention such as an admission for assesment

2. Arrange appointment with family and Jack. The assessment will be carried out by seeing Jack on is own , then with parents and sister and interviewing parents on their own and together.

3. Child observation in home and school. Interaction with parents and other school children.

Assessment will be multimodal taking into account various sources of information across different settings.

4. Observation of parent-parent and parent child interactions: Communication, boundaries, emotional atmosphere of family.

5. Assessment for causes of behaviour

Onset duration and progress of behaviour and impairment across emotional, social and equational domains.

Simple misbehaviour : Teachers may be able to differentiate and rating scales will help.

Disruptive behaviour disorder: Traits of oppositional defiant disorder and conduct disorder. Persistent aggressive, antisocial and defiant behaviour.

Disinhibited attachment disorder :distractible or restless, with attentional problems and social disinhibition in context of emotional and parental neglect

Generalised anxiety disorder : can present as disruptive but social disinhibition and impulsiveness is less likely.

ADHD:

- Inattention: careless with detail, is easily distracted, loses things
- Hyperactivity / impulsivity : fidgety, leaves seat when sitting for a long time, interrupts others, talks excessively in social context , ails to wait in queue.
- Parents should be asked to complete a rating scale such as the Conners 3-P (Conners 2008), the CADDRA ADHD Checklist or SNAP-IV-C (Swanson 2007).

Autism spectrum disorders: Language difficulties, stereotypes and rituals and impaired social interaction. Difficult to differentiate and hence rating scales such as Autism diagnostic interview and Autism Diagnostic Observation Scale may be required.

Juvenile Bipolar disorder: More likely if family history of bipolar , severe sleep disturbance .

6. Assessment of co-morbidity

- Dyscalculia
- Dyslexia
- Dyspraxia (developmental coordination disorder) and dysgraphia
- Depression
- Sleep disorders
- Specific language impairment
- Tic or Tourette's syndrome
- Intellectual disability : specific phenotypes such as Fragile X syndrome
- Assessment of cognitive functioning and patterns of behaviour is needed to identify other developmental disorders (such as intellectual disabilities, speech and language disability, global or specific learning disabilities, developmental co-ordination disorder, autism and related spectrum disorders, and Tourette syndrome).

7. Etiological risk factors in a developmental and history

- features of ADHD in relatives (including parents)

- mother's alcohol, nicotine or illicit drug intake during pregnancy
- Complication sin pregnancy or delivery
- early birth or low birth weight
- failure to regain birth weight in the first 2 weeks of life
- adoption after 6 months of age
- Temperamental characteristics
- Illnesses
- Developmental milestones.
- early exposure to environmental lead
- early closed head injury with sustained loss of consciousness
- persistent lack of sleep
- dietary problems
- ongoing family discord

8. Educational and social history : longitudinal academic performance, relationships with teachers and children. schools attended or changed, disciplinary problems.

9. Physical examination: Involve a paediatrician

Record and chart baseline height and weight. Cardiovascular examination. Observe for co-ordination abnormalities and tics.

10. Investigations may be required if Iron deficiency anaemia, thyroid dysfunction suspected. MRI is head injury suspected or focal neurological signs identified.

CHILDHOOD ADHD MANAGEMENT

After the assessment you diagnose him with ADHD.

Outline the strategies for management.

1. **A multimodal approach** is recommended for treatment of ADHD. This may include medication, psychosocial management strategies and, where appropriate, educational interventions.

2. Psychoeducation to families

3. Behaviour modification

Behaviour modification (also known as contingency management) uses strategies such as structured reward systems, response costs and discipline techniques to encourage behaviour change. Rewards can include social rewards such as approval and praise, concrete rewards such as extra recreation time, or more complex reward schemes that involve “tokens” (stars or points) that can be earned. Discipline techniques can include verbal reprimands or time-out from positive reinforcement strategies. The time-out strategies involve removing the individual from a reinforcing situation for a set period of time following inappropriate behaviour.

4. **CBT:** CBT combines cognitive therapy, which aims to modify or eliminate unwanted thoughts or beliefs, and behavioural therapy, which aims to bring about changes in behaviour. The goal of CBT is to enhance functioning at home and at school.

5. Parental handling

Parenting classes:

- A child with ADHD requires more than standard parenting if he is to learn and prosper
- It is particularly hard to be an effective parent of a child with ADHD, as the ADHD works against the child's ability to learn and the parent's sense of competence.

Recommendations:

- Set a small number of household rules, couched in language that is specific and understandable by all ('Schoolbag always packed before going to bed')
- Get beyond simply trying to control disobedience, overexcitement or hyperactivity and set out to have opportunities for a good time with their child, showing him love and appreciation
- keep verbal instructions short and clear
- include positive comments, praise and encouragement, linked with a points incentive system where appropriate
- consider deals ('Get your homework done and OK'd by me, then you can go out'), but ensure that the child's action precedes the reward or the parent's half of the deal
- use a simple disciplinary system such as '1-2-3 Magic' (www.123magic.com)
- avoid harsh punishment, harsh judgements and condemnations
- keep the child's social life alive, but arrange brief play dates with structured activities to avoid overexcited mayhem or fights
- keep as calm as possible
- plan ahead to avoid trouble.

6. Social skills training

Children with ADHD can experience difficulties in their social competence, peer relationships and family interactions. The aim of social skills training is to develop and reinforce the use of appropriate social skills.

7. Parenting programs

Parenting programs (often called parent-training programs) aim to teach parents/caregivers strategies for managing disruptive behaviour in their child and improving parent-child relationships. Parenting programs make use of behaviour modification techniques such as structured reward systems and discipline techniques.

6. Medication

Stimulants (methylphenidate or dexamfetamine) are the mainstay of medication in ADHD, particularly following the initial findings of the Multimodal Treatment of Attention Deficit Hyperactivity Disorder (MTA) study.

Methylphenidate is recommended by NICE as an initial agent (NICE 2008), although some individuals will respond better to dexamfetamine.

A physical examination must be carried out before prescribing. Potential adverse effects on cardiovascular functioning (very mildly raised blood pressure and pulse rate) mean it is also necessary to check that there are no significant cardiac symptoms and no family history of sudden cardiac death in early adulthood. Monitor weight, growth, pulse, BP and sleep.

Children who are anxious, have tics or preexisting sleep problems, or who need treatment that covers early mornings and evenings will probably need atomoxetine.

Clonidine and bupropion is also used but seek second opinion.

Reference: Attention-deficit hyperactivity disorder in children and adolescents: assessment and treatment

CAUSES OF POOR SCHOOL PERFORMANCE

You work in a Child and Adolescent Outpatient Clinic and a paediatrician has sent you a referral. 10-year-old boy attends with his mother. He lives at home with parents and siblings. The paediatrician has diagnosed him with ADHD and Tourette's and has prescribed him methylphenidate and clonidine with improvement of behaviours at home. His school performance has, however, deteriorated.

Question

What are some of the possible reasons for his poor school performance?

Here I will outline general causes. The question as you notice states that the behavioural disturbance is school specific therefore it is unlikely to be due to the medication or ADHD. Therefore weigh on other causes.

1. Ongoing ADHD due to inadequate dosage to help with increased demands at school. Is the child at home in the evening when the medication wears off (Immediate release) whilst the side effects of medication lead to poor school performance.
2. OCD symptoms, stand alone or medication induced exacerbated under school stress.
3. Sensory impairment : Visual and Auditory impairment
4. Epilepsy
5. Organic issues such as Vitamin deficiencies , Vit D, B12 , Folate
6. Learning difficulties: Dyslexia, Dysgraphia, dyscalculia, speech difficulties
7. Autism leading to poor social interaction at school
8. Emotional issues: Chronic neglect, abuse, parental dysfunction, bullying at school.
9. Poor sleep

10. Onset of depression.

11. Poor teaching i.e pedagogy , lack of support from teachers and school for special needs

ADDRESSING MOTHER'S CONCERNS ABOUT MEDICATION

Mother reports she has read in a magazine article that stimulants cause growth retardation and “drug addicts”. The child has a diagnosis of Tourette's and ADHD.

Question: How do you respond to mother's concern about medication?

1. Arrange a meeting and establish therapeutic alliance
2. Acknowledge concerns and advise that this is a common concern parents have.
3. Psychoeducation about Tourette's and ADHD
4. Advise the mother that stimulants can lead to stunted growth but this is something that will be monitored closely as will other side effects. Growth parameters will be assessed at baseline and monitored every 3-6 months. This will include monitoring of weight, height and growth velocity.
5. If growth retardation does become an issue then medication can be prescribed at low doses only or medication holidays can be considered or medication can be switched to non-stimulants.
6. Explain to the mother that there is no evidence that prescription of stimulants causes drug addiction. Untreated ADHD can be associated with a range of psychiatric issues later in life including drug misuse. Treatment of ADHD is associated with improved outcomes such as better school performance and social outcomes.

" Overall, the results of the present study show that the long-term outcomes for participants with ADHD when left untreated were poor compared with non-ADHD controls, and that treatment of ADHD improved long-term outcomes, but usually not to the point of normalization. The outcomes that were studied (with ADHD symptoms deliberately excluded as an outcome) most often included drug use/addictive behavior, academic, and antisocial behavior. This was followed by social function, self-esteem, occupation, driving, services use, and obesity outcomes". Shaw, M., Hodgkins, P., Caci, H., Young, S.,

Kahle, J., Woods, A. G., & Arnold, L. E. (2012). A systematic review and analysis of long-term outcomes in attention deficit hyperactivity disorder: effects of treatment and non-treatment. BMC medicine, 10(1), 99.

7. Advise mother about paediatrician involvement and role of school.

TOURETTE'S SYNDROME

You continue to prescribe him his medication and his school performance improves. However his mother is concerned about on-going tics.

Question: Outline your management of his Tourette's.

1. Educating the parents about Tourette's syndrome : Multiple Motor and One or More Vocal tics persistent for more than a year with onset before age of 18.
2. Education of the school about Tourette's : Explain that stress can exacerbate tics and hence some restructuring of school environment and hence flexibility from teachers is required.
3. General stress reduction and boredom reduction strategies
4. Explain that stimulants can increase the risk of tics but prolonged administration can result in reduction , therefore consider a trial for 3 - 6 months.
5. Paediatric and Neurological evaluation if the tics do not resolve after a fixed period: alternative causes of tics such as PANDAS, organic brain disorder, epilepsy , therefore MRI may be required.
6. Treat OCD with SSRI's if present as OCD and tourette's co-exist.
7. Clonidine can be used if not already prescribed in mild to moderate tics. 0.1-0.3 mg.
8. Haloperidol and Risperidone may be needed in case of non -response.
9. Address self esteem, treat depression with CBT.

MANAGEMENT OF PATIENT SUICIDE ON INPATIENT UNIT

You are the consultant of an inpatient psychiatric ward. Your patient is a 51-year-old man with psychotic depression. He appears to be improving with medication so his nursing observations are downgraded. When you arrive to work early Monday morning, you are informed that your patient has committed suicide over the weekend. He was found hanging in his bedroom on the ward.

Question:

What is your immediate response to this situation? (This question has been covered previously so only the main points will be listed)

1. Clarify situation and ensure that adequate procedures were followed: Police informed, body isolated and documentation carried out. Has a IIMS (Incident information management system) report been filled out?
2. Inform clinical director and Nurse Unit manager: Attempt to arrange a meeting with both to initiate the leadership involved, and to mobilise the resources
3. Inform family and address their concerns. Allow them to ask questions, vent anger and hostility and provide support to the family in the form of counselling services etc.
4. Notes to be copied and sent to coroner
5. Staff debriefing
6. Patient debriefing with all leave for patients reviewed especially high risk patients.
7. Seek support for self with supervisor
8. Inform medico-legal insurance
9. Root Cause analysis.
10. Longer term consider case conference or grand round presentation as part of Quality improvement

BREAKING BAD NEWS IN CHILDREN

Your patient's ex-wife calls you. She has been informed by the police of the death. She asks for advice on what to tell their 10-year-old son. He last saw his father 2 months ago. She is not sure whether to tell him about the suicide.

Question

Outline your response

1. Ethical Dilemma : Risk of psychological harm to child on informing vs withholding information. Therefore the information if conveyed needs to be conveyed in an appropriate setting and with appropriate psychological support.
2. Clarify mother's concerns by arranging a meeting : What was child's relationship with father? Was it ambivalent? How often did father see him? Was he aware of father's illness? What does he know about father's illness?
3. Guide the mother through options to help her make an informed decision with a risk benefit analysis of each option
4. Risk of not telling the child now may prevent immediate psychological distress but may affect the mother-child relationship in the future as it may raise issues of trust.
5. Speak to a Child and Adolescent psychiatrist for advice.
6. Arrange a meeting with mother and child with psychologist or other support staff who can provide after meeting support.
7. Use General Principles of Breaking Bad news in Children

SPIKES (Six step protocol for delivery of bad news)

Setting up the interview: privacy, involve significant others, make a connection, manage time constraints and interruptions)

Assess child's **P**erception : The child may know more than adults think they do.

Obtain child's **I**nvitation: How would you like to give you information?

Giving **Knowledge** and information to the child by using appropriate language, avoiding jargon, checking understanding and using audio-visual aids if necessary

Address patients **Emotions** with empathic responses : advise that they are not to blame.

Summarise.

OR

When - decide on appropriate time to break bad news

Who - significant others

Where - private place , manage interruptions etc.

What child knows already: Specialist support, truthful words " I'm afraid I'll have to tell you something very sad" Explain that death was not their fault. Explain slowly, pictures if necessary, repeat information and check understanding.

STAFF RESPONSES AFTER PATIENT SUICIDE

During the week you notice that the nursing staff are reluctant to downgrade nursing observations or to let inpatients go on leave. They request more nursing staff as they report they are unable to provide adequate nursing care.

Question

a. What are the possible reasons why the nursing staff are behaving like this?

What are the key strategies in managing this?

a.

Patient factors:

- High risk patients
- Severely unwell
- high rates of self harm and violence

Staff factors :

- Normal response to critical incident: change in behaviour to prevent future occurrences
- Fear of being blamed or scapegoated
- Lack of support or psychological debriefing
- Lack of leadership from consultant psychiatrist on the ward
- Countertransference and transference issues: Fear of being wrong, tolerance for uncertainty etc.
- Uncertainty about the fear of procedure after suicide (not knowing if they have to attend an inquiry, what process to follow etc.)

System Factors :

- Culture of blame
- Poor staffing
- Poor ward environment with high risk structures that can increase risk of a repeat event.
- Absence of an adequate response from management to staff concerns
- Poor training in risk assessment and management of agitated patients e.g tranquillisation policies.

b.

1. Enable **professionals to recognise**, understand, accept, and express emotions felt for the deceased patient (sorrow, guilt, anger, wrath, disappointment, compassion, relief etc.)
2. Enable **professionals to recognise**, understand, accept, and express **emotions felt towards him/herself** as the person involved in the treatment (disappointment, doubt, uncertainty, incompetence, fear, shame, anxiety etc.)
3. Enable the **professionals to recognise how feelings about loss are handled**(eg.denial or dissociation of feelings or, conversely, being flooded with overwhelming affect).
4. Enable **professionals to verbalise and reduce the fear of legal action** or inquest on behalf of relatives, advocates, coroners and discuss ways of minimising this risk. Explain the procedures as uncertainty of the aftermath can create fear.
5. Enable **professionals to realise the limited control the therapist has** over the patient's behavior and life and develop a tolerance for ambiguity.
6. Enable professionals to recognize the likelihood that, at least in the short term, the loss is likely to affect their work with subsequent patients as well as professional identity

PTSD IN A 49 YEAR OLD INDIGENOUS FEMALE

You are the consultant of a community mental health clinic. Your patient is a 49 year old Indigenous woman who has been case managed and treated for symptoms of PTSD for the last 6 months following a sexual assault 2 months. On her way to her appointment today, she saw her assailant, who is a distant relative, on the bus and became very distressed. Her case manager has just approached you with concerns about the level of her distress and risk of self-harm.

Question

Before assessing your patient, what information do you need from the case manager?

For this question please go through the principles on assessment in aboriginal population.

The question consists broadly of awareness of a risk assessment, cultural factors and engagement issues. I will outline the main points only

1. Risk assessment : previous risk of self harm and suicide attempts, acute suicidal ideation, access to weapons. You can add additional points from the HCR but only relevant ones

2. Cultural factors: Cultural elements required for the interview, language, explanatory models of illness, belief in traditional healing, is a indigenous liaison worker required? Please add the key issues in assessment in indigenous populations.

3. Incident of trauma : Details if known to avoid re-traumatisation during the interview later.

4. Clinical Factors: Symptoms of PTSD, Depression, Psychosis and Substance Use disorder.

5. Engagement and therapeutic alliance: assess for stigma, attitudes to mental health services, family attitudes to mental health services, compliance with medication

6. Psychosocial factors: Social supports, confidentiality issues, psychological support received financial, relationship and accomodation status

INDIGENOUS PSYCHIATRY

You see your patient with her case manager. She reports after speaking to you she feels much better. She denies any self-harm ideation and does not want to be admitted to hospital. She would like to go home and would like something to “make me feel better and help me sleep tonight”. (This is from a previous exam and from candidates memory. The question feels a bit vague)

Question

What is your immediate management plan?

1. Recognise that symptoms of psychiatric illness may not be volunteered readily and therefore high index of suspicion is required.

2. Clarify the complaint:

Is there evidence of depression, anxiety or psychosis. Are there neurovegetative symptoms present.

Are there nightmares leading to insomnia? Hypervigilance etc.

Type of insomnia : early, middle or early morning awakening.

Substances used

3. Psychoeducation to patient: Explain that sleep disturbance may signify another issues that may require antidepressant or anti anxiety treatment. PTSD may need treatment. Explain the risk of dependence with benzodiazepine and risk of weight gain and metabolic dysfunction with Quetiapine.

3. Medication intervention:

Small dose of benzodiazepine such as clonazepam 0.5 mg nocte restricted supply can be given for few days OR Quetiapine 25 mg nocte. Ensure restricted supply.

4. Offer psychological support taking into account cultural factors.

SEXUAL ASSAULT AND CONFIDENTIALITY - INDIGENOUS CASE

A week later the case manager approaches you and informs you that your patient is feeling much better after speaking about her symptoms. Your patient would like to disclose the sexual assault to her adult daughter. However she is concerned that her adult son, and other members of the community will find out.

Question: What is your response to the case manager?

1. Ethical Issues :

- Autonomy: Patient has the right to make decisions
- Confidentiality and Privacy - She wishes to protect the information from certain members of the community and family.
- Risk of harm on disclosure: Can the patient be harmed following disclosure due to ostracisation, community backlash etc. Expulsion or rejection
- Justice: Wider issues about fair treatment of individual and a community as a whole
- Mandatory reporting obligations : Are there children involved? Is the perpetrator a potential risk to children? Is there an obligation to report to police or Child protection services?

2. Discuss with the director or in peer review

3. Arrange meeting with patient with mental health case worker or such trusted health professional. Clarify concerns and situation. What does she think will happen? Explore legal options. Provide information about supports available if there is a risk of community backlash.

4. The aim is to help the patient make an informed decision after carrying out a **risk benefit analysis** of all the options.

5. Explore options and the individuals that can be involved:

- Community leaders / elders
- Traditional healers
- Indigenous mental health liaison worker
- If patient agrees to disclose then arrange a meeting with daughter and patient with key mental health professionals including female staff. Ensure psychological and social support is available after meeting.

- Offer follow up meetings for daughter to ask questions , clarify situation etc.